The Menopause Diet for Busy Women

Easy Recipes, a 12-Week Meal Plan, and Simple Daily Practices to Support Weight Management, Energy, and Wellbeing

[Author Name]

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Recipes have been written with care, but individual tolerances vary. Please check every ingredient against your own allergies and intolerances.

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A Letter to You

Dear reader,

If you have picked up this book, there is a good chance something has changed. Maybe you are waking at 3 a.m. for no obvious reason. Maybe your periods have started to behave like they have a mind of their own, or have stopped altogether. Maybe the jeans that fitted perfectly last spring now sit a little tighter around the middle, even though you have not changed a thing. Maybe you have cried in the car, snapped at someone you love, or walked into a room and forgotten why you were there, and wondered quietly whether something is wrong with you.

Let me say this as clearly as I can: **you are not broken. You are changing.**

Menopause is a natural stage of life that every woman who lives long enough will reach. Yet for generations it has been whispered about, joked about or simply ignored. Many women arrive at their forties and fifties with almost no idea what to expect, and with even less practical guidance on what actually helps. If you have felt confused, dismissed or alone, that is not a personal failing. It is a gap in the information we have been given.

This book is here to help fill that gap.

In the pages ahead, you will find a clear, kind explanation of what is happening in your body during perimenopause, menopause and the years beyond. You will learn how food can support your energy, sleep, mood and comfort, and how to care for your bones, heart, muscles and brain for the long term. You will also find realistic tools for the busiest of lives: quick recipes, simple snacks, a 12-week meal plan, grocery lists and a 30-day plan of yoga, strength and calming practices that need nothing more than a little floor space.

Here is what this book will never do:

- **It will never shame you.** There are no "good" or "bad" foods here, no punishing rules and no before-and-after pressure. Your worth has nothing to do with the number on a scale.

- **It will never promise miracles.** Food is powerful, but it is not magic. Some symptoms respond beautifully to changes in what and when you eat. Others need medical support, and that is perfectly okay. You will find honest information about both.
- **It will never ask you to do it all at once.** Small, steady changes made on ordinary days are what make the difference. You can start with a single breakfast.

You deserve to feel strong, comfortable and like yourself again, and you deserve to enjoy the food on your plate while you get there. Wherever you are in this transition, whether you are just noticing the first signs or you are years beyond your final period, this is a wonderful time to start looking after yourself with the same care you have probably given to everyone else.

Make a cup of tea, find a comfortable chair and let's begin.

With warmth,

[Author Name]

How to Use This Book

This book is designed to be used, not just read. It is organized so you can dip in wherever you need to, on whatever kind of day you are having. There is no single right way through it, but here are three easy ways in.

Three Ways to Begin

1. Read it cover to cover. If you like to understand the "why" before the "how," start with Part One. It explains each stage of the menopause transition in plain language, so every later recommendation makes sense. Part Two looks at treatments, therapies and the everyday habits with the strongest evidence. Part Three turns all of that into practical, symptom-focused nutrition.

2. Jump straight to your symptom. If hot flashes, broken sleep or afternoon energy crashes are your biggest concern right now, go straight to Chapter 10, "Eating for Your Symptoms." Each symptom has its own section, with what may help, what may make things worse and a "Try This Week" box with one or two simple actions. You will also find a symptom index at the back of the book.

3. Start the plan today. If you are ready to get going, read Chapter 8, "Not Sure of Your Stage? Start Here," and Chapter 12, "The Busy Woman's Kitchen," then turn to the 12-week meal plan in Part Five. You can come back to the background chapters whenever you are curious.

How the Book Fits Together

Part	What you will find	Best for
Part One	What is happening in your body and mind at each stage	Understanding and reassurance
Part Two	Treatments, therapies and everyday habits	Informed conversations with your doctor

Part	What you will find	Best for
Part Three	The Menopause Plate, symptom-by-symptom nutrition, weight and kitchen skills	Daily eating decisions
Part Four	30 main recipes and 25 simple snacks	Cooking
Part Five	The 12-week meal plan with grocery lists and prep plans	Structure and routine
Part Six	The 30-day yoga, strength and calm plan	Movement, stress and sleep

Features You Will See

Throughout the book, a few recurring features help you find the most useful information quickly:

- **Chapter Summary** boxes recap the key points at the end of each chapter.
- **Try This Week** boxes suggest one or two small, specific actions to test.
- **What the Evidence Says** boxes explain where the science is strong and where it is still emerging.
- **Good to Know** boxes add practical tips, safety notes or reminders.
- **Checklists** give you something simple to tick off, print or photograph.

Recipe Labels

Every recipe carries simple labels, so you can see at a glance whether it suits your needs.

Label	Meaning
GF	Gluten-free (or "GF option" with a simple swap)
DF	Dairy-free (or "DF option")
V	Vegetarian
VG	Vegan (or "VG option")
Make-Ahead	Keeps well for batch cooking or freezing
30 min	Ready in 30 minutes or less (the quickest recipes show their exact time, e.g., "10 min")

A Gentle Note on Pace

You do not need to change everything this week. In fact, it works better if you don't. Choose one thing that feels doable, such as adding protein to breakfast or drinking a glass of water before your morning coffee, and let it become routine before you add the next. Progress, not perfection, is the goal.

> **PLEASE KEEP YOUR DOCTOR IN THE LOOP**
>
> This book offers general, evidence-informed guidance. It cannot know your personal health history. If you have a medical condition, take regular medication, are considering hormone therapy or supplements, or have symptoms that worry you, talk to your doctor or another qualified healthcare professional. A good clinician is one of the best allies you can have during this transition.

A Note on the Science

Menopause research has grown enormously in the past twenty years, and it is still evolving. Some recommendations rest on large, well-designed studies that have been repeated many times. Others are based on promising early findings that need more research before we can be confident. A few popular claims have very little evidence behind them at all.

Throughout this book, we have tried to be honest about which is which. Where it matters, you will see two simple labels:

- **Established:** supported by strong, consistent evidence, such as large clinical trials, systematic reviews or the guidelines of major health organizations. You can feel confident acting on these.
- **Emerging:** supported by early or mixed research. These ideas are interesting and may be worth trying if they are safe for you, but the evidence is not yet settled.

Where Our Information Comes From

The guidance in this book draws on the published recommendations of respected organizations, including The Menopause Society (formerly the North American Menopause Society), the UK's National Institute for Health and Care Excellence (NICE), the National Health Service (NHS), the US National Institutes of Health (NIH), the World Health Organization (WHO) and leading bone, heart and cancer research organizations, as well as peer-reviewed scientific studies. A list of references and trusted resources appears at the back of the book.

What This Means for You

Science gives us averages. You are an individual. A change that helps most women may not help you, and something with only emerging evidence may make a real difference in your life. The best approach is to stay curious, try one change at a time, notice how you feel, and talk to a healthcare professional about anything that affects your medical care.

Health advice also changes as new research appears. If you read something here that differs from what your doctor tells you, trust the person who knows your history, and feel free to bring this book along to your next appointment.

PART ONE

Understanding Your Changing Body

Before we talk about food, let's talk about you: what is happening, why it is happening, and why none of it is your fault.

CHAPTER 1

The Menopause Transition, Explained Simply

Understanding what is happening inside your body is the first step to feeling at home in it again.

For something that happens to half the world's population, menopause is remarkably misunderstood. Many of us grew up thinking of it as a single moment, the day periods stop, usually somewhere around fifty. In reality, the menopause transition is a process that unfolds over several years, sometimes more than a decade, and it can begin long before most women expect.

This chapter lays the foundation for everything else in the book. Once you understand what your hormones are doing and why, the nutrition and lifestyle advice in later chapters will make much more sense, and many of the confusing things you may be experiencing will finally have an explanation.

What Menopause Actually Is (and Isn't)

Strictly speaking, **menopause is a single point in time**: the day that marks twelve months since your last menstrual period. It is a moment you can only identify by looking back. Everything before it is called **perimenopause**, and everything after it is **post-menopause**.

In everyday conversation, though, most people use "menopause" to describe the whole transition, and that is perfectly fine. In this book, we will use the specific terms when the difference matters and the general term when it doesn't.

One day in time vs. a transition of several years

Here is a useful way to picture it. Imagine your reproductive life as a long road. For most of your adult years, the road is fairly smooth: hormones rise

and fall in a predictable monthly rhythm. Perimenopause is the stretch where the road becomes bumpy and unpredictable. Menopause is the signpost you pass when you have gone a full year without a period. Post-menopause is the new, smoother road beyond it.

The bumpy section is where many women experience the most symptoms, and it lasts on average around four years, although for some women it is much shorter and for others it stretches to ten years or more.

Average age, natural, surgical and medically induced menopause

In the United States, the average age of natural menopause is around 51 to 52, and most women reach it somewhere between 45 and 55. Your genes play a part: women often reach menopause at a similar age to their mothers and sisters. Smoking tends to bring it forward by one to two years.

Not every menopause arrives naturally, though:

- **Natural menopause** happens gradually as the ovaries slowly wind down their production of reproductive hormones.
- **Surgical menopause** happens immediately when both ovaries are removed (an oophorectomy), sometimes alongside a hysterectomy. Because hormone levels drop suddenly rather than gradually, symptoms can be abrupt and intense.
- **Medically induced menopause** can follow certain cancer treatments, such as chemotherapy or pelvic radiation, or medications that deliberately suppress the ovaries. It may be temporary or permanent.

A hysterectomy that removes the uterus but leaves the ovaries in place ends periods but does not, on its own, cause menopause. The ovaries keep working, although some women reach menopause a little earlier afterward. Without periods as a guide, it can simply be harder to tell when the transition is happening, so symptoms become the main clue.

Early, Surgical and Medical Menopause: When Your Timeline Is Different

Around 1 in 100 women experience menopause before the age of 40. This is called **premature ovarian insufficiency (POI)**. A further 5 in 100 or so reach menopause between 40 and 45, which is known as **early menopause**.

If this is you, please know that you are not alone, and that your experience deserves particular care. Reaching menopause early means living longer without the protective effects of estrogen on your bones, heart and brain. For this reason, medical guidelines generally recommend that women with POI or early menopause speak with a doctor about hormone therapy, usually at least until the average age of natural menopause, unless there is a medical reason not to. This is very different from the conversation women have at 55 or 60.

An early or sudden menopause can also bring grief: for fertility, for the timeline you imagined, or simply for feeling "too young" for this. Those feelings are valid. Chapter 5 talks more about the emotional side of the transition, and support groups for women with POI can be a lifeline.

> **WHEN TO SEE A DOCTOR SOONER**
>
> If you are under 45 and your periods have become irregular or stopped, or you have symptoms such as hot flashes and night sweats, it is worth seeing your doctor. Early menopause is diagnosed differently from menopause at the usual age, and it often benefits from specific treatment.

Meet Your Hormones

Hormones are chemical messengers. They travel through your bloodstream and tell different parts of your body what to do. During the menopause transition, several of them change at once, and because they influence almost every system in the body, the effects can show up in surprising places.

Estrogen

Estrogen is the hormone most closely linked with menopause. It is made mainly by the ovaries during your reproductive years, and it does far more than regulate periods. Estrogen helps to:

- keep bones strong by slowing the rate at which old bone is broken down
- keep blood vessels flexible and support healthy cholesterol levels
- regulate body temperature through its effects on the brain
- support mood, memory and sleep
- keep skin, vaginal and bladder tissues plump, moist and elastic
- influence where the body stores fat

After menopause, the ovaries produce very little estrogen. The body still makes small amounts in fat tissue and other places, but levels are much lower than before.

Progesterone

Progesterone is produced after ovulation each month. It prepares the lining of the uterus for a possible pregnancy and has a naturally calming, sleep-supporting effect on the brain. In perimenopause, ovulation becomes less regular, so progesterone is often the first hormone to decline. Lower progesterone can contribute to heavier or irregular periods, poorer sleep and feeling more anxious or on edge.

Testosterone

Women make testosterone too, in the ovaries and adrenal glands. It contributes to libido, energy and muscle. Testosterone declines gradually with age, starting well before menopause, rather than dropping suddenly at the transition.

FSH: the messenger that shouts louder

Follicle-stimulating hormone (FSH) is released by the pituitary gland in the brain. Its job is to tell the ovaries to mature an egg each month. As the ovaries become less responsive, the brain "shouts louder," releasing more FSH to try to get a response. That is why FSH levels tend to rise during the transition. However, FSH can swing up and down from month to month in perimenopause, which is why a single blood test is often unhelpful (more on this in Chapter 3).

Cortisol: the stress hormone in the background

Cortisol is not a reproductive hormone, but it matters here. It is released in response to stress and helps regulate blood sugar, blood pressure and your sleep–wake cycle. Poor sleep, hot flashes in the night and the pressures of midlife can all keep cortisol running high, which may in turn affect appetite, belly fat storage and mood. The good news is that cortisol responds well to everyday habits such as regular meals, movement, rest and calming practices, which you will find throughout this book.

Why Fluctuation, Not Just Decline, Drives Many Symptoms

It is tempting to think of perimenopause as a slow, steady slide in hormones. In reality, it is more like a rollercoaster. During perimenopause, estrogen can surge to higher levels than you had in your thirties one month and plunge the next. Ovulation may happen some months and not others.

These sharp ups and downs are behind many of the most frustrating symptoms of perimenopause: unpredictable periods, breast tenderness, migraines, mood swings and sleep that is fine one week and terrible the next. Many women find that once they are through to post-menopause and hormone levels settle at a new, steady low, some of these symptoms ease, even though others, such as vaginal dryness, may continue.

Understanding this can be very reassuring. If your symptoms seem to come and go with no pattern, it is not your imagination, and it is not because you are doing something wrong. Your body is adjusting to a changing hormonal landscape.

The Three Stages at a Glance

	Perimenopause	Menopause	Post-menopause
What it is	The transition years before the final period	The point 12 months after your last period	All the years that follow
Typical timing	Often begins in the 40s; lasts around 4 years on average, sometimes 10 or more	Average age 51 to 52	From menopause onward
Hormones	Fluctuating, sometimes wildly	Estrogen and progesterone now low	Settled at a new, low level
Periods	Irregular, heavier, lighter or skipped	None for 12 months	None (any bleeding needs checking)
Common focus	Cycle changes, sleep, mood, hot flashes, energy	Hot flashes, sleep, joints, intimacy, body composition	Bone, heart, muscle and brain health; lingering symptoms

> **BLEEDING AFTER MENOPAUSE**
>
> Any vaginal bleeding that happens after you have gone 12 months without a period should always be checked by a doctor promptly. It is often caused by something harmless, but it needs to be investigated.

Why Every Woman's Experience Is Different

Some women sail through the transition with barely a hot flash. Others find their lives turned upside down. Most are somewhere in between. Roughly three in four women experience hot flashes and night sweats at some point, and around a quarter find their symptoms severe enough to have a significant effect on daily life.

Genetics, lifestyle, stress, culture and health history

Many factors shape how you experience menopause:

- **Genetics.** The age at which your mother reached menopause is a reasonable clue to your own timing.
- **Health history.** Conditions such as thyroid disease, diabetes, migraines or a history of depression can affect how you feel during the transition.
- **Lifestyle.** Smoking, heavy drinking, very low activity levels and poor sleep can all make symptoms harder to manage.
- **Body weight.** Women with a higher body weight often report more hot flashes in perimenopause.
- **Stress and life circumstances.** Midlife often brings caring for children and aging parents, career pressure and relationship changes at the same time as hormonal change. Stress does not cause menopause, but it can amplify how you feel.
- **Attitudes and expectations.** Research suggests that the way a culture views aging and menopause can influence how women experience it.

Not One-Size-Fits-All: Culture, Ethnicity and Your Menopause

One of the largest long-term studies of women in midlife, the Study of Women's Health Across the Nation (SWAN), followed thousands of women of different ethnic backgrounds in the United States for more than two decades. It found meaningful differences between groups. On average, Black women

reported hot flashes for longer, often around ten years, than white women, while women of Japanese and Chinese heritage reported fewer hot flashes overall. Black and Hispanic women also tended to reach menopause slightly earlier.

These are averages, not predictions, and the reasons are complex. Genetics, diet, stress, access to healthcare and the effects of discrimination all play a part. What matters for you is this: **your experience is valid, even if it doesn't look like what you have read about or what your friends describe.** If you feel that your symptoms are being dismissed, Chapter 6 offers practical tips for advocating for yourself at medical appointments.

Food traditions matter too. The recipes in this book draw on Mediterranean, South Asian, East Asian, Latin American and West African kitchens, because nourishing food looks different around the world, and there is no single "right" menopause diet.

CHAPTER SUMMARY

- Menopause is technically one day: the point 12 months after your final period. Perimenopause comes before it, post-menopause after it.
- The average age of natural menopause is 51 to 52. Menopause before 45 is early, and before 40 is called premature ovarian insufficiency; both deserve medical attention.
- Estrogen affects bones, heart, brain, temperature, mood, skin and fat storage, which is why symptoms can appear almost anywhere.
- In perimenopause, it is the wild swings in hormones, not just the decline, that drive many symptoms.
- Every woman's transition is different. Genetics, health, lifestyle, stress and culture all shape your experience.

CHAPTER 2

Perimenopause: The Shifting Years

The years before your final period can be the most confusing of all. Knowing what to look for changes everything.

Perimenopause is the part of the transition that most women know least about, and yet it is often the stage with the most noticeable symptoms. It usually begins in your forties, although for some women it starts in their late thirties. Because periods are still happening, many women (and, frustratingly, some healthcare providers) don't connect new symptoms with hormonal change at all.

If you have been told "you're too young for menopause" while lying awake drenched in sweat, this chapter is for you.

Early Signs You Might Be in Perimenopause

There is no single test that tells you perimenopause has started. Instead, it is usually recognized by a pattern of changes. Some of the most common early signs include:

Cycle changes

- Periods that come closer together (for example, every 21 to 24 days instead of every 28)
- Periods that are further apart, or skipped months
- Bleeding that is heavier, lighter, longer or shorter than you are used to
- Worse premenstrual symptoms than before

A change in the length of your cycle by seven days or more, persisting over several months, is one of the classic early markers of perimenopause. Later in the transition, gaps of 60 days or more between periods become common.

Sleep disruption

Many women notice their sleep changing before anything else. You may find it harder to fall asleep, wake more often during the night, or wake very early

and be unable to drift off again. Night sweats can play a part, but sleep can also become lighter even without them, partly because of falling progesterone.

Anxiety and mood changes

New or worsening anxiety, irritability, low mood and a feeling of being overwhelmed are very common in perimenopause and are often among the first signs. Chapter 5 explores this in depth.

Heavier or lighter periods

Fluctuating estrogen without the balancing effect of regular progesterone can cause the uterine lining to build up more than usual, leading to heavier bleeding or flooding. Other women find their periods become lighter and shorter. Both can be normal parts of perimenopause, but very heavy bleeding deserves attention (see "When to Talk to Your Doctor" below).

Common Physical Changes

As the transition progresses, other symptoms often appear. You might experience some, many or very few of them.

- **Hot flashes and night sweats.** A sudden wave of heat, usually in the face, neck and chest, sometimes with flushing, sweating, a racing heart and then chills. When they happen at night, they are called night sweats. These are known medically as *vasomotor symptoms*.
- **Breast tenderness.** Higher-than-usual estrogen peaks can make breasts sore or swollen.
- **Headaches and migraines.** Women who had hormone-related migraines in the past may find they become more frequent during perimenopause, before often improving after menopause.
- **Bloating and digestive changes.** Hormonal fluctuations affect fluid balance and the gut, so bloating, constipation or a more sensitive stomach are common.
- **Joint aches.** Stiff, achy joints, especially in the morning, are surprisingly common.
- **Weight and shape changes.** Many women notice weight gathering around the middle, even without changes to how they eat. Chapter 11 explains why.

Why hot flashes happen

Your brain has an internal thermostat in an area called the hypothalamus. Estrogen helps keep this thermostat stable. As estrogen fluctuates and falls, the comfortable temperature zone narrows, so even a tiny rise in core body temperature can trigger the body's cooling response: blood vessels near the skin widen, you flush and sweat, and your heart rate rises. Researchers have identified a group of brain cells (called KNDy neurons) that become overactive when estrogen is low, which has led to a new class of non-hormonal treatments described in Chapter 6.

The Surprising Symptoms No One Warned You About

Because estrogen has receptors throughout the body, perimenopause can show up in ways most women have never heard of. These symptoms are real, they are common, and they are worth mentioning to your doctor:

- **Heart palpitations.** A fluttering, pounding or racing heartbeat, sometimes during a hot flash. Most of the time they are harmless, but palpitations should always be checked by a doctor to rule out other causes, especially if you also feel dizzy, breathless or have chest pain.
- **Itchy, dry or crawly skin.** Estrogen helps skin hold onto moisture and collagen. Some women experience itching or a sensation like insects crawling on the skin (called *formication*).
- **Tinnitus.** Ringing or buzzing in the ears is reported by some women during the transition.
- **Frozen shoulder.** Pain and stiffness in the shoulder joint that limits movement is most common in women aged 40 to 60, and hormonal change may be one reason why.
- **Dry eyes and dry mouth.**
- **Tingling in the hands or feet, or a burning feeling in the mouth.**
- **New allergies or sensitivities.**

None of these mean that something is seriously wrong, but each deserves to be taken seriously and assessed by a healthcare professional.

Energy, Cravings and the Blood-Sugar Connection

Many women describe a new kind of tiredness in perimenopause: not just feeling sleepy, but feeling drained, foggy and flat. There are several reasons.

Disrupted sleep is an obvious one. Heavy periods can lower iron levels. Stress keeps the body on high alert. And changing estrogen levels can affect how sensitive your body is to insulin, the hormone that moves sugar from your blood into your cells.

When your body becomes a little less sensitive to insulin, meals that are high in quickly digested carbohydrates (such as white bread, pastries, sweetened drinks or sugary cereals) can lead to a bigger rise in blood sugar followed by a sharper dip. That dip often shows up as an energy crash, irritability ("hangry" feelings) and strong cravings for something sweet, which can set the cycle going again.

The good news is that this is one of the areas where food can make the biggest difference. Meals that combine protein, fiber and healthy fats are digested more slowly and help keep energy steady. You will find this principle at the heart of the Menopause Plate in Chapter 9 and throughout the recipes.

What Your Body Needs Most in This Stage

Perimenopause is the time to start building habits that will serve you for decades. A few nutrients deserve special attention now:

Nutrient	Why it matters in perimenopause	Good food sources
Iron	Heavy periods can deplete iron, leading to fatigue, breathlessness and hair shedding	Lean red meat, poultry, fish, lentils, beans, tofu, spinach, fortified cereals
B vitamins	Support energy production and the nervous system	Whole grains, eggs, dairy, legumes, leafy greens, meat and fish
Magnesium	Involved in muscle relaxation, sleep and blood-sugar control	Pumpkin seeds, almonds, cashews, spinach, black beans, whole grains, dark chocolate
Protein	Keeps you full, steadies blood sugar and protects muscle	Eggs, Greek yogurt, fish, poultry, tofu, tempeh, beans, lentils
Fiber	Supports digestion, fullness and steady energy	Vegetables, fruit, oats, beans, lentils, whole grains, seeds

A note on iron

If your periods are heavy, it is worth asking your doctor to check your iron levels, including ferritin (your iron stores). Low iron is very common in perimenopause and easy to miss. Pairing iron-rich plant foods with a source of vitamin C, such as peppers, tomatoes, citrus or berries, helps your body absorb more of the iron. Tea and coffee can reduce iron absorption, so try to drink them between meals rather than with them. Please don't take iron supplements without checking with a doctor first, since too much iron can be harmful.

Is It Perimenopause, or Something Else?

Many perimenopause symptoms overlap with other common health conditions. This is one reason it is so important not to assume that everything is "just hormones," and equally important not to be told that everything is "just stress."

Thyroid changes, low iron and depression: overlapping signs worth checking

- **Thyroid problems.** An underactive thyroid (hypothyroidism) is common in midlife women and can cause tiredness, weight gain, low mood, dry skin, hair thinning and changes to periods. An overactive thyroid can cause anxiety, palpitations, sweating and sleep problems. A simple blood test can check your thyroid function.
- **Low iron or anemia.** Causes fatigue, breathlessness, dizziness, feeling cold and hair loss.
- **Depression and anxiety disorders.** These can occur at any age, and the risk of depression rises during perimenopause, particularly in women with a history of depression, postnatal depression or severe premenstrual symptoms.
- **Low vitamin D or vitamin B12.** Both can cause tiredness, low mood and aches.
- **Diabetes.** Can cause tiredness, thirst and frequent urination.
- **Sleep apnea.** Becomes more common after menopause and can cause exhaustion, morning headaches and poor concentration.

It is entirely possible to be in perimenopause *and* have one of these conditions at the same time. A good doctor will consider both.

When to Talk to Your Doctor

You do not need to wait until your symptoms are unbearable. It is always reasonable to talk to a healthcare professional if your symptoms are affecting your quality of life. In particular, make an appointment if you notice:

- ☐ Very heavy bleeding (soaking through a pad or tampon every hour or two, passing large clots, or bleeding that lasts more than seven days)
- ☐ Bleeding between periods or after sex
- ☐ Periods that come more often than every 21 days
- ☐ Symptoms of perimenopause before the age of 45
- ☐ Palpitations, especially with dizziness, chest pain or breathlessness
- ☐ Low mood, anxiety or thoughts of self-harm (seek help urgently)
- ☐ Symptoms that are disrupting your sleep, work or relationships

For women over 45 with typical symptoms, perimenopause is usually diagnosed from your symptoms and cycle pattern alone. Blood tests are generally not needed and can be misleading because hormone levels change so much from day to day. They may be useful in specific situations, for example if you are under 45 or your doctor wants to rule out other conditions.

TRY THIS WEEK

- Start a simple symptom diary. Each evening, note your cycle day, sleep quality, energy, mood and any hot flashes. After four to six weeks, patterns often start to appear, and the diary will make your doctor's appointment far more productive. (Appendix A has a tracker you can use.)
- Add a palm-sized portion of protein to your breakfast at least four days this week.

CHAPTER SUMMARY

- Perimenopause usually begins in your forties and lasts around four years on average, though it can be shorter or much longer.
- Early signs include changes to your cycle, disrupted sleep and new anxiety or low mood.
- Hot flashes are caused by a narrowing of the brain's comfortable temperature zone as estrogen fluctuates.
- Less familiar symptoms, such as palpitations, itchy skin, tinnitus and frozen shoulder, are real and worth discussing with a doctor.
- Steady meals with protein, fiber and healthy fats help protect your energy and reduce cravings.
- Iron, B vitamins, magnesium, protein and fiber deserve extra attention now.
- Some conditions, such as thyroid problems, low iron and depression, mimic perimenopause. It is worth checking.

Perimenopause Checklist

- ☐ I have started tracking my cycle and symptoms.
- ☐ I know which symptoms I would like to discuss with my doctor.
- ☐ I have asked about checking my iron (ferritin) levels if my periods are heavy.
- ☐ I am including protein at every meal.
- ☐ I am aiming for vegetables, fruit, beans or whole grains at most meals.
- ☐ I have one small, calming routine I can turn to on difficult days.

CHAPTER 3

Menopause: Reaching the Milestone

Twelve months without a period marks a turning point. For many women, it is also the start of feeling more like themselves again.

Menopause, the moment when you have gone twelve consecutive months without a period, is a milestone that passes quietly. There is no ceremony, and you only know you have reached it in hindsight. Yet the years around this point are often when symptoms such as hot flashes and sleep problems are at their strongest, and when longer-term changes to metabolism, bones and the heart begin to accelerate.

This chapter looks at what happens around the time of menopause and how to look after yourself through it.

How Menopause Is Confirmed

For most women over 45, menopause is confirmed simply by looking back: if you have not had a period for twelve months, and there is no other explanation such as pregnancy, certain medications or a medical condition, you have reached menopause.

Why blood tests are not always needed or reliable

Many women expect a blood test to give them a clear answer. In practice, guidelines such as those from NICE advise that blood tests for FSH are usually *not* needed to diagnose perimenopause or menopause in women over 45 with typical symptoms. The reason is that hormone levels can swing so much during the transition that a single result may be misleading. One month your FSH may look "menopausal"; the next it may be back in the normal range.

Blood tests can be helpful in certain situations, for example:

- if you are under 45 and your doctor suspects early menopause or premature ovarian insufficiency
- if you use hormonal contraception (such as an IUD) that stops your periods, which makes it harder to know where you are in the transition
- if your doctor wants to rule out other causes of your symptoms, such as thyroid disease

If you use hormonal contraception, ask your doctor when and how it is safe to stop, since you can still become pregnant during perimenopause. Contraception is generally advised until one year after your last period if you are over 50, or two years if you are under 50.

The Symptoms That May Peak Now: Hot Flashes, Sleep and Joints

For many women, the year or two either side of the final period is when vasomotor symptoms (hot flashes and night sweats) are most frequent and intense. Research from the SWAN study found that hot flashes last a median of around seven years in total, and they can continue for more than ten years in some women. That is far longer than the "few months" many women are told to expect, so if yours are lingering, you are in good company.

Sleep often suffers at the same time. Night sweats wake you; anxiety keeps you awake; you may need the bathroom more often during the night. Over weeks and months, poor sleep affects mood, appetite, concentration and resilience. Chapter 7 and Chapter 10 include practical strategies for better nights, and the evening routines in Part Six are designed to help you wind down.

Joint pain and stiffness are also common around menopause, often in the hands, knees, hips and shoulders. Estrogen has anti-inflammatory effects and helps maintain cartilage and the fluid that cushions joints, so its decline can leave joints feeling achy. Gentle, regular movement, strength work and an anti-inflammatory eating pattern (Chapter 9) can all help.

Desire, Comfort and Connection: Your Sexual Health

This is a topic many women find hard to raise, even with their doctor. Yet changes to sexual health are among the most common effects of menopause, and unlike hot flashes, they often don't improve on their own over time. You

deserve to feel comfortable in your body, and there are effective treatments available.

Libido, vaginal dryness and comfortable intimacy

As estrogen falls, the tissues of the vulva, vagina and bladder become thinner, drier and less elastic. This is called **genitourinary syndrome of menopause (GSM)**, and it affects around half of postmenopausal women, possibly more. Symptoms may include:

- vaginal dryness, itching or burning
- discomfort or pain during sex
- needing to urinate more often or more urgently
- more frequent urinary tract infections

Desire can change too. Lower hormones play a part, but so do tiredness, stress, body image, relationship dynamics and simply not feeling like yourself. Low libido is common and nothing to be ashamed of.

What helps:

- **Vaginal moisturizers**, used regularly (two or three times a week), can relieve everyday dryness.
- **Lubricants** used during sex make intimacy more comfortable. Water-based and silicone-based options are widely available.
- **Low-dose vaginal estrogen**, available as a cream, tablet or ring, is very effective for GSM and is absorbed into the bloodstream only in very small amounts. It is described in more detail in Chapter 6.
- **Pelvic floor physical therapy** can help with pain, tension and bladder symptoms.
- **Open conversation** with a partner, and with your doctor, can make an enormous difference.

The Metabolism Shift and Changes in Body Composition

Many women notice that their body seems to "work differently" around menopause. You may be eating the same foods and moving the same amount, yet your weight creeps up or your shape changes. This is not your imagination, and it is not a lack of willpower.

Several things are happening at once:

- **Muscle naturally declines with age**, and the loss tends to speed up around menopause. Because muscle is metabolically active tissue, less muscle means your body uses slightly less energy at rest.
- **Fat distribution shifts.** With less estrogen, the body tends to store more fat around the abdomen (visceral fat) rather than on the hips and thighs.
- **Insulin sensitivity can decline**, making blood-sugar control a little harder.
- **Sleep and stress** affect hunger hormones, making you more likely to feel hungry and to crave energy-dense foods.

Large studies suggest that the overall slowdown in metabolism with age is smaller than many people think, and that changes in muscle, activity and sleep are bigger factors. That is encouraging, because those are things you can influence. Building and protecting muscle through strength training and enough protein is one of the most effective things you can do for your metabolism, your blood sugar and your long-term health. Chapter 11 covers this in detail, with no crash diets in sight.

Skin, Hair and Nails: What's Changing and What Helps

Estrogen supports collagen, the protein that gives skin its structure and bounce. In the first five years after menopause, skin can lose up to a third of its collagen. You may notice that your skin is drier, thinner, more sensitive or slower to heal, and that fine lines appear more quickly.

Hair can change too. Some women notice thinning on the scalp, while others find new hair growth on the chin or upper lip, as the balance between estrogen and testosterone shifts. Nails may become more brittle.

What helps:

- **Protect your skin from the sun.** Daily broad-spectrum sunscreen is the most effective anti-aging step there is.
- **Moisturize generously**, ideally after bathing, with fragrance-free products if your skin is sensitive.
- **Eat enough protein** to give your body the building blocks for skin, hair and nails.
- **Include healthy fats**, such as olive oil, avocado, nuts, seeds and oily fish, which support the skin's natural barrier.

- **Eat plenty of colorful fruit and vegetables**, which provide vitamin C (needed to make collagen) and protective antioxidants.
- **Stay hydrated.**
- **Don't smoke**, as smoking significantly speeds up skin aging.
- **Talk to your doctor** about significant hair loss. It can have other causes, including low iron and thyroid problems, which are easy to test for.

What Your Body Needs Most in This Stage

Nutrient	Why it matters now	Good food sources
Protein	Helps protect muscle as it becomes harder to maintain; supports skin, hair and nails	Fish, poultry, eggs, Greek yogurt, cottage cheese, tofu, tempeh, edamame, beans, lentils
Calcium	Bone loss speeds up in the years around menopause	Dairy, calcium-set tofu, fortified plant milks, canned fish with bones, kale, bok choy, almonds
Vitamin D	Needed to absorb calcium; supports muscle strength	Sunlight, oily fish, eggs, fortified foods; many people need a supplement (ask your doctor)
Fiber	Supports gut health, steady blood sugar, fullness and healthy cholesterol	Vegetables, fruit, oats, barley, beans, lentils, whole grains, nuts, seeds
Phytoestrogen-rich foods	May modestly reduce hot flashes for some women	Soy foods (tofu, tempeh, edamame, soy milk), ground flaxseed, chickpeas, lentils

Your calcium needs increase to around 1,200 milligrams a day from age 51, according to the US National Institutes of Health. Food is the best source; if you think you may not be getting enough, speak to your doctor before taking a supplement. Chapter 9 explains how to meet your needs with everyday foods.

SOY AND HOT FLASHES

Emerging. Several reviews suggest that soy isoflavones may modestly reduce how often hot flashes happen, by around a fifth on average, although results vary from study to study. Some women seem to benefit more than others, possibly because of differences in gut bacteria. A recent small trial combining a plant-based diet with half a cup of cooked soybeans a day reported large reductions in hot flashes, but larger studies are still needed. Soy foods are safe and nutritious for most women, so they are worth including if you enjoy them. Chapter 9 looks at soy and breast cancer, a common concern.

TRY THIS WEEK

- Choose one calcium-rich food to add to your day, such as a glass of fortified soy milk, a pot of yogurt or a portion of calcium-set tofu.
- Keep a water-based lubricant and a vaginal moisturizer at home, even if you don't need them yet. Discomfort is far easier to address early.

CHAPTER SUMMARY

- Menopause is confirmed after 12 months without a period. Blood tests are usually not needed if you are over 45 and have typical symptoms.
- Hot flashes often peak around the final period and last around seven years on average, sometimes longer.
- Vaginal dryness, discomfort and bladder changes (GSM) are very common, don't usually improve on their own, and respond well to treatment.
- Changes in weight and shape are driven largely by muscle loss, fat redistribution, sleep and blood-sugar changes, not a lack of willpower.
- Protein, calcium, vitamin D, fiber and phytoestrogen-rich foods are key priorities now.

Menopause Checklist

- ☐ I know the date of my last period, or have noted it in my tracker.
- ☐ I have talked to my doctor about contraception, if relevant.

- ☐ I have mentioned any vaginal, bladder or sexual health concerns to my doctor.
- ☐ I include a calcium-rich food at least twice a day.
- ☐ I include protein at every meal.
- ☐ I protect my skin with daily sunscreen.
- ☐ I am moving my body most days and doing some strength work twice a week.

CHAPTER 4

Post-Menopause: Thriving Beyond

Life after menopause is not an ending. It is a new chapter, and it can be one of the most confident, energetic and freeing of all.

Once you have passed the twelve-month milestone, you are post-menopausal for the rest of your life. For many women, this stage brings a welcome sense of calm. Periods are gone, hormone levels have settled, and the unpredictable ups and downs of perimenopause are behind you. Many women describe a renewed sense of freedom and clarity in their fifties and sixties.

At the same time, life with lower estrogen brings some long-term changes that are worth understanding, particularly for your bones, heart, muscles and brain. The good news is that these are also the areas where daily habits, especially nutrition and movement, have the most powerful effect.

Which Symptoms Ease and Which May Linger

Every woman is different, but in general:

- **Symptoms that often ease over time:** mood swings linked to hormonal fluctuation, breast tenderness, hormone-related migraines, and, for most women, hot flashes and night sweats (although these can continue for many years).
- **Symptoms that may continue or appear later:** vaginal dryness and discomfort, bladder changes and urinary tract infections, joint stiffness, dry skin and changes in sleep.

If any symptom continues to affect your quality of life, it is worth discussing with your doctor. There is no age limit on feeling comfortable.

Protecting Your Bones

Bone is living tissue. Throughout life, your body constantly breaks down old bone and builds new bone. Estrogen helps keep this process in balance.

When estrogen falls, bone is broken down faster than it is rebuilt, and bone density begins to decline.

Osteoporosis risk and what helps

Women can lose up to 20 percent of their bone density in the five to seven years following menopause. Over time, this can lead to **osteoporosis**, a condition in which bones become fragile and more likely to break. According to bone health organizations, around one in two women over 50 will break a bone because of osteoporosis during their lifetime. Fractures of the hip, spine and wrist are the most common, and hip fractures in particular can seriously affect independence.

Osteoporosis is often called a "silent" disease because there are no symptoms until a bone breaks. That is why prevention matters so much, and why it is never too late to start.

What helps:

- **Weight-bearing and strength exercise.** Bones respond to load. Walking, dancing, stair climbing and, especially, strength training send signals to your bones to stay strong. Studies of supervised, progressive strength training in postmenopausal women with low bone density have shown improvements in bone density and physical function.
- **Calcium.** Aim for around 1,200 mg a day, mainly from food.
- **Vitamin D.** Helps you absorb calcium. Many people, especially those who get little sun, need a supplement. Your doctor can advise you.
- **Protein.** Around half of bone volume is made of protein, and adequate protein supports bone strength, particularly alongside enough calcium.
- **Not smoking, and limiting alcohol.** Both speed up bone loss.
- **Fall prevention.** Balance training, good lighting at home and suitable footwear all reduce the risk of fractures.
- **Medical treatment when needed.** Hormone therapy helps prevent bone loss, and there are several effective medications for osteoporosis. A bone density scan helps your doctor decide what is right for you.

Protecting Your Heart

Heart disease is the leading cause of death in women, and the risk rises after menopause. Before menopause, estrogen helps keep blood vessels flexible and cholesterol levels favorable. Afterward, LDL ("bad") cholesterol tends to

rise, blood pressure often creeps up, and the body becomes more prone to storing fat around the abdomen, which is linked to heart disease and type 2 diabetes.

Cholesterol, blood pressure and lifestyle

Women's heart attack symptoms can be different from the "classic" crushing chest pain. They may include breathlessness, nausea, unusual tiredness, discomfort in the jaw, neck, back or arm, or a feeling of indigestion. If you experience these, seek urgent medical help.

What helps your heart:

- **An eating pattern rich in plants**: vegetables, fruit, beans, lentils, whole grains, nuts and seeds. Mediterranean-style eating has some of the strongest evidence for heart health.
- **Fiber**, particularly soluble fiber from oats, barley, beans and lentils, which helps lower LDL cholesterol.
- **Healthy fats** such as extra virgin olive oil, nuts, seeds, avocado and oily fish, in place of saturated fats from processed meats, fried foods and pastries.
- **Less salt**, mainly by cooking more at home and choosing fewer processed and takeout foods, to help manage blood pressure.
- **Regular movement**: around 150 minutes of moderate activity a week, such as brisk walking.
- **Not smoking**, limiting alcohol, managing stress and sleeping well.

Muscle, Balance and Healthy Aging

From around the age of 30, we gradually lose muscle mass and strength. This process, called **sarcopenia** when it becomes significant, speeds up around menopause. Muscle matters for far more than appearance. It helps you get up from a chair, carry shopping, play with grandchildren, keep your balance and stay independent. It also helps control blood sugar and supports bone health.

The two most effective ways to protect muscle are:

1. **Strength training** at least twice a week. This can be bodyweight exercises, resistance bands, free weights or machines. Part Six includes two short strength sessions a week that need no equipment.

2. **Enough protein**, spread throughout the day. Many experts now suggest that older adults aim for around 1.0 to 1.2 grams of protein per kilogram of body weight per day, and possibly more if you are very active or recovering from illness, with around 25 to 30 grams at each meal. Chapter 9 shows what that looks like on your plate.

Balance naturally declines with age too, but it responds quickly to practice. Simple exercises like standing on one leg while brushing your teeth, and the balance poses in the yoga plan, can make a real difference.

Brain Health and Staying Sharp

Many women worry about memory during the menopause transition. The brain fog of perimenopause is real, but research suggests that for most women it is temporary, with memory and concentration often improving after menopause.

Looking further ahead, women are more likely than men to develop dementia, and researchers are actively studying the role of hormones. While there is still much to learn, the habits that protect the heart also protect the brain:

- regular physical activity
- a Mediterranean-style eating pattern rich in vegetables, berries, leafy greens, beans, nuts, olive oil and fish
- good sleep
- managing blood pressure, blood sugar and cholesterol
- staying socially connected and mentally engaged
- protecting your hearing (hearing loss is a recognized risk factor for dementia)
- not smoking and limiting alcohol

What Your Body Needs Most in This Stage

Nutrient	Why it matters now	Good food sources
Vitamin B12	Absorption declines with age and with some medicines (such as metformin and acid-reducing medications); low levels cause fatigue and nerve problems	Fish, meat, eggs, dairy, fortified plant milks and cereals, nutritional yeast

Nutrient	Why it matters now	Good food sources
Calcium	Supports bone strength	Dairy, calcium-set tofu, fortified plant milks, canned fish with bones, leafy greens
Vitamin D	Helps absorb calcium; supports muscle	Oily fish, eggs, fortified foods; often a supplement
Omega-3 fats	Support heart and brain health	Salmon, sardines, mackerel, trout, walnuts, ground flaxseed, chia seeds
Protein (a little more)	Helps protect muscle and bone with age	Spread across all meals and snacks

Your iron needs fall after menopause, from around 18 mg to 8 mg a day, because you are no longer losing blood each month. Unless your doctor advises otherwise, most post-menopausal women do not need an iron supplement, and many multivitamins designed for women over 50 contain little or no iron for this reason.

Know Your Numbers: The Health Checks Worth Asking For

Midlife is a great time to get a clear picture of your health. Knowing your numbers lets you and your doctor spot changes early, when they are easiest to address.

Bone density scan, blood pressure, cholesterol and blood sugar

Check	Why it matters	How often (general guide)
Blood pressure	High blood pressure rarely causes symptoms but raises the risk of heart disease and stroke	At least once a year, or as advised
Cholesterol (lipid panel)	LDL cholesterol often rises after menopause	Every few years, or more often if raised
Blood sugar (HbA1c or fasting glucose)	Detects prediabetes and diabetes early	Every few years from midlife, or as advised
Bone density (DXA) scan	Measures bone density and fracture risk	Recommended from 65, or earlier if you have risk factors

Check	Why it matters	How often (general guide)
Waist measurement	A simple marker of abdominal fat and metabolic risk	Whenever you like, as a gentle check-in
Vitamin D, B12, thyroid	Common deficiencies and conditions in midlife	If you have symptoms or risk factors
Cancer screening	Breast, cervical and colorectal screening save lives	As recommended by your national screening program

Earlier bone density testing is often advised if you went through menopause early, have a low body weight, smoke, have a parent who broke a hip, take long-term steroid medication, or have had a fracture from a minor fall. Recommendations vary between countries, so ask your doctor what is right for you.

TRY THIS WEEK

- Book a check-up if you have not had your blood pressure and cholesterol checked in the past year.
- Practice standing on one leg for 30 seconds on each side while you brush your teeth, holding the counter lightly for support if needed.

CHAPTER SUMMARY

- Post-menopause is a new, settled stage of life. Some symptoms ease; others, like vaginal dryness, may continue and are very treatable.
- Bone loss speeds up after menopause. Strength training, calcium, vitamin D, protein and fall prevention are your best protection.
- Heart disease risk rises after menopause. Plant-rich eating, fiber, healthy fats, movement and not smoking all help.
- Muscle is one of your greatest assets for healthy aging. Protect it with strength training and enough protein.
- Vitamin B12, calcium, vitamin D, omega-3s and protein deserve particular attention, while iron needs fall.
- Know your numbers: blood pressure, cholesterol, blood sugar and, when appropriate, bone density.

Post-Menopause Checklist

- ☐ I know my most recent blood pressure, cholesterol and blood sugar results.
- ☐ I have asked my doctor whether I need a bone density scan.
- ☐ I am doing strength exercises at least twice a week.
- ☐ I include protein at every meal and aim for around 25 to 30 grams.
- ☐ I eat oily fish or plant sources of omega-3s regularly.
- ☐ I have asked whether I need vitamin D or B12 checked or supplemented.
- ☐ I am up to date with breast, cervical and bowel cancer screening.

CHAPTER 5

Mind and Mood: The Emotional Side of Menopause

If you have felt unlike yourself, you are not imagining it, and you are certainly not alone.

Ask women what surprised them most about perimenopause and many will not mention hot flashes. They will talk about anxiety that arrived out of nowhere, sudden rage over small things, crying in the supermarket, or a flatness that made it hard to enjoy anything. They will talk about forgetting words mid-sentence and wondering whether something was seriously wrong.

These emotional and cognitive changes are among the most common, and most commonly dismissed, parts of the menopause transition. This chapter is here to validate them, explain them and point you toward help.

Anxiety, Low Mood and Irritability: Why They Happen

Estrogen and progesterone have powerful effects on the brain. Estrogen influences serotonin, dopamine and other chemical messengers involved in mood, motivation and emotional resilience. Progesterone has a calming effect, partly through a substance it produces called allopregnanolone, which acts on the same brain receptors as some anti-anxiety medications.

When these hormones fluctuate and fall, the brain has to adjust. For some women, this shows up as:

- new or worse anxiety, sometimes with a racing heart or a constant sense of dread
- irritability and a much shorter fuse
- low mood, tearfulness or a loss of joy
- loss of confidence and self-esteem
- feeling overwhelmed by things that used to feel manageable

Research shows that the risk of depression increases during perimenopause, even in women who have never been depressed before. Women with a history of depression, postnatal depression or severe premenstrual mood symptoms (PMDD) may be more sensitive to these hormonal changes.

Hormones are only part of the story. Poor sleep, hot flashes, chronic stress and the many demands of midlife all affect mood. So does how we feel about aging and our changing bodies. It is usually a combination of factors, which is good news, because it means there are several ways to help.

When Big Feelings Feel Bigger: Rage, Tears and Overwhelm

Many women describe a kind of anger in perimenopause they have never felt before: sudden, intense and out of proportion to whatever triggered it. Others find themselves in tears several times a day. These experiences can be frightening and can leave you feeling guilty or ashamed afterward.

Please be gentle with yourself. These reactions are often a sign that your nervous system is running on empty, from hormonal shifts, sleep deprivation and stress all at once. Some things that may help in the moment:

- **Pause and breathe.** Slow your breathing, making your out-breath longer than your in-breath (for example, in for four, out for six). This activates the body's calming response. The breathing practices in Part Six are designed for exactly these moments.
- **Name it.** Simply saying to yourself, or to someone close, "this is my hormones talking, and I need a minute" can take the heat out of a situation.
- **Step away.** A short walk, a glass of cold water or a few minutes alone can help the wave pass.
- **Eat something.** Low blood sugar can make anger and tears much worse. A snack with protein can help steady things surprisingly quickly.
- **Repair afterward.** Everyone loses their temper sometimes. A simple apology and explanation is enough.

If rage or tears are frequent, intense or affecting your relationships, please talk to your doctor. Both hormonal and non-hormonal treatments can help.

Brain Fog and Memory Lapses

Forgetting names, losing your train of thought, walking into rooms and forgetting why, struggling to concentrate at work: brain fog is one of the most common and distressing symptoms of the transition. Many women quietly fear early dementia.

Here is the reassuring news. Research, including findings from the SWAN study, suggests that changes in memory and processing speed during perimenopause are generally mild and temporary, and that they tend to improve after menopause. Brain fog is also strongly linked to poor sleep, hot flashes, stress and low mood, all of which can be treated.

What helps:

- **Prioritize sleep** wherever you can (Chapter 7).
- **Keep blood sugar steady** with regular, balanced meals (Chapter 10).
- **Stay hydrated.** Even mild dehydration can affect concentration.
- **Move.** Exercise increases blood flow to the brain and supports memory.
- **Use external memory aids** without guilt: lists, calendar reminders, a notebook by the bed. This is not a failing; it is good strategy.
- **Do one thing at a time** where possible. Multitasking becomes harder when you are tired.

If memory problems are getting steadily worse, affect your ability to manage daily life, or are noticed by others, see your doctor to rule out other causes.

Identity, Body Image and Self-Compassion

Menopause can stir up big questions. Who am I now? What does it mean to no longer be able to have children? Why don't I recognize the woman in the mirror? Our culture often links women's value to youth and appearance, so it is no wonder that many women feel a sense of loss during this time.

At the same time, many women describe midlife as a time of liberation: caring less about what others think, knowing what matters, and having the confidence to ask for it.

Self-compassion is one of the most useful tools for this stage of life. It means treating yourself with the same kindness you would offer a good friend. Research suggests that self-compassion is linked to lower anxiety and

depression, and to healthier eating and exercise habits, because people tend to look after things they value.

A few ways to practice:

- Notice your inner voice. If you would not say it to a friend, try not to say it to yourself.
- Speak about your body in terms of what it does for you, not only how it looks.
- Replace "I should" with "I would like to" or "I'm choosing to."
- Spend time with people who make you feel good about yourself.
- Reflect on what you want this next chapter of your life to hold.

Relationships, Work and Talking About Menopause

Menopause affects not only you but also the people around you. Partners may not understand why you are tired, irritable or less interested in sex. Children may notice your shorter fuse. Colleagues may not realize that a hot flash in a meeting is exhausting, or that you were awake half the night.

Talking about what you are experiencing can ease a lot of pressure. You don't have to share everything, but simple explanations help others understand and support you:

- *"I'm going through perimenopause. My sleep is really disrupted at the moment, so I'm more tired than usual."*
- *"When I get a hot flash, I just need a few minutes and some fresh air."*
- *"I'm not angry with you. My hormones are making everything feel more intense right now, and I'm working on it."*

At work, many employers now have menopause policies or support. Practical adjustments can make a big difference: a desk fan, access to cold water, flexible start times after a bad night, the option to work from home, or breathable uniforms. If you feel comfortable, speak to your manager or human resources team about what would help.

When to Seek Support

Emotional symptoms deserve the same care as physical ones. Effective support is available, including hormone therapy, cognitive behavioral

therapy (CBT), antidepressants, counseling and peer support. Your doctor can help you decide what suits you.

Red flags and where to turn

Please seek help promptly if you:

- feel persistently low, hopeless or numb for more than two weeks
- have lost interest or pleasure in most things
- are struggling to function at work or at home
- are using alcohol or other substances to cope
- have panic attacks
- have thoughts of harming yourself, or feel that others would be better off without you

IF YOU ARE IN CRISIS

If you are having thoughts of suicide or self-harm, please reach out for help immediately. Contact your local emergency number, go to your nearest emergency department, or call a crisis line. In the United States, you can call or text **988** to reach the 988 Suicide & Crisis Lifeline at any time. In the UK and Ireland, you can call Samaritans on **116 123**. You do not have to face this alone.

TRY THIS WEEK

- Each evening this week, write down one thing you are grateful for and one thing you did well today, however small.
- Practice the "longer out-breath" for two minutes, twice a day: breathe in for a count of four and out for a count of six.

CHAPTER SUMMARY

- Anxiety, irritability, low mood, tearfulness and brain fog are common, real effects of the menopause transition.
- Hormonal changes affect brain chemicals involved in mood, but sleep, stress and life circumstances matter too.
- Rage and tears are often signs of an overloaded nervous system. Breathing, stepping away, eating and repairing afterward all help.
- Brain fog is usually mild and temporary, and improves with better sleep, steady blood sugar and movement.
- Self-compassion supports both mental health and healthy habits.
- Help is available. Seek support promptly if your mood is persistently low or you are having thoughts of self-harm.

Reflection Prompts

Take a few quiet minutes with a notebook and consider:

1. Which emotional changes have I noticed in the past year?
2. What situations or times of day make them better or worse?
3. Who could I talk to about what I am experiencing?
4. What is one thing I could take off my plate this month?
5. What do I want this next chapter of my life to feel like?

PART TWO

Treatments, Therapies & Lifestyle

You have more options than ever before. Knowing them helps you make the choices that are right for you.

CHAPTER 6

Treatments and Therapies: Knowing Your Options

Information is power. The more you know about your options, the better the conversations you can have with your doctor.

Nutrition and lifestyle can make a real difference to how you feel during the menopause transition, and the rest of this book will show you how. But for many women, food and exercise alone are not enough, especially for moderate to severe hot flashes, night sweats, mood changes or vaginal symptoms. There is no prize for suffering in silence, and choosing medical treatment is not a failure.

This chapter offers a balanced, evidence-informed overview of the main treatments available, so you can talk them through confidently with your healthcare provider. It is not a substitute for personal medical advice: the right choice for you depends on your symptoms, health history, preferences and values.

> **HOW TO READ THIS CHAPTER**
>
> Throughout, we label evidence as **established** (supported by strong, consistent research and major guidelines) or **emerging** (promising, but not yet settled). Treatments and guidelines change as research develops, and availability and brand names differ between countries.

Menopausal Hormone Therapy (MHT/HRT)

Menopausal hormone therapy, often called hormone replacement therapy (HRT), replaces some of the hormones your ovaries no longer produce in the same amounts. It is the **most effective treatment for hot flashes and night sweats** (established), and it also helps prevent bone loss and fractures.

A brief history: why so many women are wary

In 2002, early results from a large American study called the Women's Health Initiative (WHI) were widely reported as showing that hormone therapy increased the risk of breast cancer, heart disease and stroke. Prescriptions dropped dramatically around the world almost overnight.

Since then, much more careful analysis has shown a more nuanced picture. The women in the WHI were, on average, 63 years old, many years past menopause, and many had existing risk factors. For healthy women who start hormone therapy before the age of 60, or within ten years of menopause, the benefits generally outweigh the risks. This is the current position of The Menopause Society and other major organizations (established).

Types, benefits, risks and who it may suit

Hormone therapy comes in several forms:

- **Estrogen** is the main ingredient that relieves symptoms. It can be taken as a tablet, or through the skin as a patch, gel or spray (called *transdermal* estrogen).
- **A progestogen** (progesterone or a similar hormone) is needed if you still have a uterus, to protect the lining of the uterus from overgrowth, which could otherwise increase the risk of endometrial cancer. This can be a tablet, part of a combined patch, or a hormonal IUD.
- Women who have had a hysterectomy can usually take **estrogen alone**.
- **Testosterone** is sometimes prescribed for women with low sexual desire that is causing distress, when other factors have been addressed.

Possible benefits:

- relief of hot flashes and night sweats, often within a few weeks
- better sleep, mood and quality of life for many women
- prevention of bone loss and reduced risk of fractures
- improvement in vaginal dryness and joint aches for some women

Possible risks and side effects:

- **Blood clots and stroke.** Estrogen taken as a tablet slightly increases the risk of blood clots. Transdermal estrogen (patches, gels or sprays) at standard doses does not appear to increase this risk in the same way (established), so it is often preferred for women with risk factors.

- **Breast cancer.** Combined hormone therapy (estrogen with a progestogen) is associated with a small increase in breast cancer risk that rises with longer use. Estrogen-only therapy appears to have little or no increase in risk. To put this in context, the increase in risk from combined therapy is similar to or smaller than the increase linked to regularly drinking two or more alcoholic drinks a day, or to obesity.
- **Side effects** such as breast tenderness, bloating, headaches or irregular bleeding are common in the first few months and often settle, or can be helped by adjusting the dose or type.

Hormone therapy is not usually recommended for women who have had breast cancer, certain types of blood clots, stroke, heart disease or active liver disease, though specialist advice may differ in individual situations.

> **HORMONE THERAPY AT A GLANCE**
>
> **Established:** Hormone therapy is the most effective treatment for hot flashes and night sweats, and it prevents bone loss. For most healthy women under 60 or within 10 years of menopause, benefits outweigh risks. Women with premature or early menopause are generally advised to take hormone therapy until at least the average age of menopause, unless there is a reason not to.
>
> **Emerging:** Whether starting hormone therapy early has lasting benefits for the heart or brain is still being studied. Hormone therapy is not currently recommended solely to prevent heart disease or dementia.

"Bioidentical" hormones: a word of caution

You may see compounded "bioidentical" hormones advertised as more natural or safer. Some regulated, approved hormone products are chemically identical to the body's own hormones (such as estradiol and micronized progesterone), and these are widely prescribed. However, custom-compounded products made in individual pharmacies are not tested and regulated in the same way, so their dose, purity and safety can vary. Major menopause organizations recommend regulated, approved products wherever possible.

Vaginal Estrogen and Genitourinary Health

For vaginal dryness, discomfort during sex and bladder symptoms (genitourinary syndrome of menopause, described in Chapter 3), **low-dose vaginal estrogen** is very effective (established). It comes as a cream, a small tablet or pessary inserted into the vagina, or a soft ring that stays in place for three months.

Because the dose is very low and acts locally, only tiny amounts reach the bloodstream. For this reason, it is considered safe for most women, can be used long-term, and does not usually require a progestogen. It can be used alongside systemic hormone therapy, and many women who cannot or prefer not to take systemic hormones can still use it. Women who have had breast cancer should discuss it with their oncology team.

Other options for genitourinary symptoms include non-hormonal vaginal moisturizers and lubricants, vaginal DHEA (prasterone), a tablet called ospemifene, and pelvic floor physical therapy.

Non-Hormonal Prescription Options

Not every woman can or wants to take hormone therapy. Fortunately, there are now several effective non-hormonal medicines for hot flashes and night sweats.

When hormone therapy isn't an option for you

Women who have had breast cancer or other hormone-sensitive cancers, a history of blood clots or stroke, or who simply prefer to avoid hormones, have several options (established unless noted):

- **Neurokinin receptor antagonists.** A newer class of medicines, including fezolinetant and elinzanetant, works directly on the overactive temperature-control cells in the brain described in Chapter 2. In clinical trials they significantly reduced the number and severity of hot flashes. Because they are new, longer-term safety data are still being gathered, and your doctor may monitor your liver function.
- **Certain antidepressants (SSRIs and SNRIs).** Low doses of medicines such as paroxetine, escitalopram, citalopram, venlafaxine and desvenlafaxine can reduce hot flashes, even in women who are not depressed. Some are also helpful for mood and anxiety. Note that some

SSRIs can interact with tamoxifen, so women taking it need specialist advice.

- **Gabapentin.** An anti-seizure and nerve-pain medicine that can reduce hot flashes, particularly at night. Drowsiness is a common side effect, which may actually help if night sweats are disrupting sleep.
- **Oxybutynin.** A bladder medicine that can also reduce hot flashes, although it may not be suitable for older women because of possible effects on memory.
- **Clonidine.** A blood pressure medicine with a modest effect on hot flashes; side effects often limit its use.

Cognitive Behavioral Therapy (CBT) for Hot Flashes and Sleep

CBT is a structured, practical form of talking therapy that helps you notice and change unhelpful patterns of thinking and behavior. CBT designed specifically for menopause does not usually reduce how *often* hot flashes happen, but it significantly reduces how much they *bother* you and interfere with daily life (established). It also helps with sleep, low mood and anxiety. NICE guidelines now recommend that CBT be offered for menopause symptoms.

CBT for insomnia (CBT-I) is the first-line recommended treatment for long-term sleep problems in adults, and it works well during the menopause transition. CBT can be accessed through a therapist, a structured group, a book or an evidence-based online program.

Clinical hypnosis, delivered by a trained practitioner or through guided recordings, has also been shown to reduce hot flashes in clinical trials and is recommended by The Menopause Society as a non-hormonal option (established).

Supplements: What's Supported, What's Unproven, What to Be Careful With

The supplement aisle is full of products promising menopause relief. Some are harmless but ineffective, some have limited evidence, and a few can interact with medications or cause side effects. "Natural" does not always mean safe, and supplements are not tested and regulated as strictly as medicines.

Supplement	What the evidence says
Soy isoflavones	Emerging: may modestly reduce hot flash frequency for some women; soy foods are a good first step
Black cohosh	Mixed and inconsistent results; rare reports of liver problems; not recommended by The Menopause Society
Red clover, dong quai, evening primrose oil, wild yam	Little or no good evidence of benefit; wild yam creams do not convert to progesterone in the body
St. John's wort	Some evidence for mood, but serious interactions with many medicines, including tamoxifen, blood thinners and oral contraceptives
Vitamin D	Established: important for bone health; many people need a supplement, especially in winter
Calcium	Best from food; supplements may be advised if your diet falls short
Omega-3 (fish oil)	Good for heart health as part of the diet; does not appear to reduce hot flashes

Creatine, collagen and magnesium: hype or help?

These three supplements are hugely popular with women in midlife. Here is where the evidence currently stands:

- **Creatine (emerging).** A well-researched supplement among athletes, creatine helps muscles produce energy during short bursts of effort. In older adults, combining creatine with strength training may lead to greater gains in muscle strength than training alone. Research in menopausal women specifically, including possible effects on bone, mood and brain function, is still limited and mixed. It appears safe for most healthy people at typical doses (around 3 to 5 grams a day), but check with your doctor if you have kidney problems.
- **Collagen (emerging, limited).** Some studies suggest collagen supplements may modestly improve skin hydration and elasticity, but many are funded by manufacturers, and higher-quality independent

research is needed. Collagen is a protein, but it lacks some essential amino acids, so it is not a substitute for protein from food when it comes to building muscle.

- **Magnesium (emerging, limited).** Magnesium is essential for muscle and nerve function, and many people don't get enough from their diet. Evidence that magnesium supplements improve sleep or menopausal symptoms is limited. A food-first approach, with nuts, seeds, beans, whole grains and leafy greens, is a safe place to start. Magnesium supplements can cause digestive upset and interact with some medicines.

Before taking any supplement, check with your doctor or pharmacist, especially if you take prescription medications, have a medical condition or have had cancer.

Feeling Unheard? How to Advocate for Yourself at Appointments

Many women say they have been dismissed, misdiagnosed or told "it's just your age." Menopause training for healthcare professionals has historically been limited, though it is improving. You deserve to be listened to, and a few simple steps can make your appointment far more productive:

1. **Prepare.** Bring your symptom diary or a list of your symptoms, how often they happen and how they affect your life. Be specific: "I wake three to four times a night drenched in sweat, and I've had to cancel work twice this month" is far more powerful than "I'm not sleeping well."
2. **Say the word.** Ask directly: "Could this be perimenopause or menopause?"
3. **Prioritize.** If time is short, start with the one or two symptoms that bother you most.
4. **Ask about options.** "What are all my treatment options, including hormonal and non-hormonal ones? What are the benefits and risks for someone like me?"
5. **Bring support.** A partner or friend can help you remember what was said.
6. **Take notes,** or ask if you can record the conversation.

7. **Ask for a follow-up** to review how a treatment is working, typically after three months.
8. **Seek a second opinion** if you feel unheard. You can ask to see a clinician with a special interest in menopause. The Menopause Society has a directory of certified menopause practitioners.

Questions to Ask Your Doctor

Photograph or copy this list to take to your next appointment.

- ☐ Are my symptoms likely to be caused by perimenopause or menopause?
- ☐ Do I need any tests, and why?
- ☐ Is hormone therapy a suitable option for me? What are the benefits and risks in my situation?
- ☐ Which type of hormone therapy would you recommend, and why?
- ☐ What non-hormonal options could help my symptoms?
- ☐ Could vaginal estrogen help with dryness, discomfort or bladder symptoms?
- ☐ Are any of my current medicines or supplements likely to affect my symptoms or interact with treatment?
- ☐ Do I need a bone density scan, or cholesterol, blood sugar or thyroid tests?
- ☐ Can you refer me for CBT, pelvic floor therapy or a menopause specialist?
- ☐ When should we review how things are going?

CHAPTER SUMMARY

- Hormone therapy is the most effective treatment for hot flashes and night sweats and protects against bone loss. For most healthy women under 60 or within 10 years of menopause, the benefits outweigh the risks.
- Transdermal estrogen carries a lower risk of blood clots than tablets. Women with a uterus need a progestogen as well.
- Low-dose vaginal estrogen is safe and effective for vaginal and bladder symptoms for most women.
- Effective non-hormonal medicines include neurokinin receptor antagonists, certain antidepressants and gabapentin.
- CBT and clinical hypnosis help with hot flashes, sleep and mood.
- Most herbal supplements have little evidence of benefit, and some carry risks. Always check with your doctor first.
- You deserve to be heard. Prepare for appointments and ask for a menopause specialist if you need one.

CHAPTER 7

Everyday Habits That Make a Difference

The most powerful medicine is often found in the small things we do every day.

Alongside nutrition, a handful of everyday habits have strong evidence for helping women feel better during the menopause transition and for protecting long-term health. None of them require perfection, expensive equipment or hours of free time. They simply work best when they become part of ordinary life.

This chapter covers movement, sleep, stress, drinks, pelvic floor health and the often-overlooked power of connection.

Strength Training Made Simple: Protecting Muscle and Bone

If you take just one habit from this chapter, make it this one. Strength training (also called resistance training) means working your muscles against resistance, whether that is your own body weight, resistance bands, dumbbells or gym machines. It is one of the most effective things you can do in midlife (established) because it:

- builds and maintains muscle, which naturally declines with age
- helps maintain bone density and reduces the risk of osteoporosis
- improves blood-sugar control and insulin sensitivity
- supports a healthy metabolism and body composition
- improves balance and reduces the risk of falls
- benefits mood, sleep and self-confidence

How much do you need?

Health guidelines, including those from the World Health Organization, recommend muscle-strengthening activities that work all the major muscle

groups on **two or more days a week**. That is it. Two sessions of 15 to 30 minutes can make a real difference.

Getting started

- **Start where you are.** Bodyweight exercises such as sit-to-stand from a chair, wall push-ups, glute bridges and step-ups are an excellent beginning. Part Six includes two 15-minute strength sessions a week that need no equipment.
- **Focus on form.** Move slowly and with control. Quality matters more than quantity.
- **Progress gradually.** When an exercise starts to feel easy, make it a little harder: add a few repetitions, slow the movement down, or add a light weight or band. Muscles and bones respond to being challenged.
- **Aim for "challenging but doable."** By the last two or three repetitions of a set, the exercise should feel hard, but you should still be able to keep good form.
- **Rest between sessions.** Muscles grow stronger during recovery, so leave a day between strength sessions for the same muscle groups.

> **SAFETY FIRST**
>
> If you have osteoporosis, a previous fracture, joint problems, high blood pressure or another health condition, check with your doctor or a physical therapist before starting a new program. They can suggest safe, effective exercises for you. Many women with osteoporosis can and should strength train, with the right guidance.

Cardio and Everyday Movement for Your Heart

Aerobic exercise, or "cardio," is any activity that gets your heart rate up and makes you breathe a little harder: brisk walking, cycling, swimming, dancing, hiking or an exercise class. It protects your heart, lowers blood pressure, improves cholesterol, supports a healthy weight, and lifts mood.

Guidelines recommend at least **150 minutes of moderate activity a week** (for example, 30 minutes on five days), or 75 minutes of vigorous activity, or a mix of both. Moderate means you can talk but not sing; vigorous means you can only say a few words before pausing for breath.

It doesn't have to happen all at once. Ten-minute walks after meals count, and walking after eating has the added bonus of helping to lower blood-sugar spikes.

The power of simply moving more

Beyond formal exercise, all the movement you do through the day adds up. Sitting for long periods is linked to poorer health, even in people who exercise. Try to:

- stand up and move for a few minutes every hour
- take phone calls while walking
- use stairs instead of elevators where you can
- park a little further away or get off the bus a stop early
- garden, clean, play with children or pets, or dance in the kitchen

Every step counts.

Sleep Hygiene for Hot Nights and Busy Minds

Sleep problems affect a large share of women during the menopause transition, and poor sleep makes almost every other symptom feel worse. While hot flashes and night sweats may need medical treatment, good sleep habits help everyone.

Keep cool

- Keep your bedroom cool, ideally around 60 to 67°F (15 to 19°C).
- Use breathable cotton, linen or bamboo bedding and nightwear, or moisture-wicking fabrics.
- Layer your bedding so you can easily throw off or add a layer.
- Keep a glass of cold water and a small fan by your bed.
- Try a cool shower before bed.

Set your body clock

- Go to bed and get up at roughly the same time every day, including weekends.
- Get outside in natural daylight in the morning, even for ten minutes. This helps set your internal clock.
- Keep naps short (under 30 minutes) and before mid-afternoon.

Wind down

- Create a calming routine for the last 30 to 60 minutes before bed: dim the lights, read, stretch, take a warm (not hot) bath, or try one of the evening practices in Part Six.
- Put screens away, or at least use night mode, an hour before bed.
- Keep a notebook by the bed and write down worries or to-do lists to get them out of your head.

Watch what (and when) you eat and drink

- Avoid caffeine after early afternoon (more on this below).
- Limit alcohol, which disrupts sleep and can trigger night sweats.
- Try not to eat a large or very spicy meal within two to three hours of bedtime.
- If you wake hungry, a small snack containing protein and complex carbohydrates, such as a little Greek yogurt with oats or a slice of whole-grain toast with nut butter, may help.

If you can't sleep

If you have been awake for around 20 minutes, get up, go to another dimly lit room and do something calm until you feel sleepy, then return to bed. This helps your brain relearn that bed is for sleep. If insomnia continues for weeks, ask your doctor about CBT for insomnia (Chapter 6), which is the most effective long-term treatment.

> **IS IT SLEEP APNEA?**
>
> Sleep apnea, in which breathing repeatedly stops and starts during sleep, becomes much more common after menopause. Signs include loud snoring, gasping or choking during the night, waking with a headache or dry mouth, and feeling exhausted despite a full night in bed. It is very treatable, so talk to your doctor if this sounds familiar.

Stress, Cortisol and the Power of Rest

Midlife is often one of the busiest, most demanding times of life. Many women are juggling work, children, aging parents, relationships and their own changing health all at once. Chronic stress keeps the body's stress hormone, cortisol, elevated, which can worsen sleep, increase cravings and

abdominal fat storage, and make hot flashes and mood symptoms harder to cope with.

You can't remove all stress from your life, but you can build in regular moments of recovery that help your nervous system reset:

- **Breathing practices.** Slow, deep breathing with a longer out-breath activates the body's "rest and digest" response within minutes.
- **Mindfulness and meditation.** Even five to ten minutes a day can reduce stress and improve mood and sleep. The 30-day plan in Part Six includes guided meditations.
- **Yoga.** Combines movement, breath and relaxation, and has been shown to improve psychological symptoms and quality of life in menopausal women.
- **Time in nature.** Walking in green spaces is linked with lower stress levels.
- **Saying no.** Protecting your time and energy is a health habit too.
- **Rest without guilt.** Rest is not laziness. It is how your body and mind repair.

Rethinking Your Drinks: Alcohol and Caffeine

Alcohol

Many women find that their relationship with alcohol changes in midlife. A glass of wine that once felt relaxing may now trigger hot flashes, disrupt sleep or leave you feeling anxious the next day. Alcohol also:

- increases the risk of breast cancer, even at low levels of drinking (established)
- contributes to bone loss and falls
- adds calories without nutrients, and can increase appetite
- can worsen mood and anxiety

There is no amount of alcohol that is completely risk-free, but if you choose to drink, keeping it to a minimum makes a difference. Current US Dietary Guidelines advise that women who drink have no more than one drink a day, and many health organizations now encourage drinking less than that. Try having several alcohol-free days each week, alternating alcoholic drinks with water, choosing smaller glasses, or exploring the growing range of alcohol-

free drinks. The Tart Cherry Evening Spritz and other "soothing sips" in Part Four make lovely alternatives.

Caffeine

Coffee and tea can be part of a healthy diet, and both contain beneficial plant compounds. However, caffeine can trigger hot flashes in some women, worsen anxiety and palpitations, and interfere with sleep, since it can stay in your system for many hours.

- Most healthy adults can safely have up to around 400 mg of caffeine a day (roughly three to four cups of brewed coffee), but many women in the transition feel better with less.
- Try to have your last caffeinated drink before about 2 p.m.
- Notice whether caffeine affects your hot flashes, sleep or anxiety, and adjust accordingly.
- Swap some cups for decaffeinated coffee, herbal teas or a Golden Turmeric Latte.
- Remember that caffeine is also found in tea, cola, energy drinks and chocolate.

Plus: support if you want to stop smoking

Smoking is linked to earlier menopause, more severe hot flashes, faster bone loss, heart disease, several cancers and faster skin aging. Stopping at any age brings real health benefits, and the risks of heart disease begin to fall soon after you quit. You are much more likely to succeed with support. Your doctor or pharmacist can recommend stop-smoking medicines, nicotine replacement and free local or national quit services.

Pelvic Floor Health

The pelvic floor is a hammock of muscles that supports your bladder, bowel and uterus. Pregnancy, childbirth, aging and lower estrogen can all weaken it. Signs of a weak pelvic floor include leaking urine when you cough, sneeze, laugh or exercise; needing to rush to the bathroom; or a feeling of heaviness in the vagina.

These symptoms are common, but they are not something you simply have to live with. Pelvic floor exercises (also called Kegels) are effective for many women (established):

1. Sit or lie comfortably. Imagine you are trying to stop yourself from passing wind and stop the flow of urine at the same time. Squeeze and lift those muscles inward and upward.
2. **Slow squeezes:** hold the squeeze for up to 10 seconds, then fully relax for 10 seconds. Repeat up to 10 times.
3. **Quick squeezes:** squeeze and lift quickly, then relax straight away. Repeat up to 10 times.
4. Aim to do a set of both, three times a day. Keep breathing normally and avoid squeezing your buttocks or thighs.

It can take three months or more to notice a difference. If symptoms persist, ask your doctor for a referral to a pelvic floor physical therapist, who can check that you are doing the exercises correctly and offer other treatments. Vaginal estrogen can also help with bladder symptoms.

Connection, Community and Purpose

Strong social connections are one of the most powerful predictors of long-term health and happiness. Loneliness, on the other hand, is linked with poorer mental and physical health. Yet midlife can be an isolating time: children leave home, careers change, relationships shift, and symptoms can make us withdraw.

Connection doesn't have to mean a busy social calendar. It might be:

- a regular walk or coffee with a friend
- joining a menopause support group, in person or online
- a class, choir, book club, volunteering or community project
- talking openly with partners, family and colleagues about how you are feeling
- reconnecting with an old friend

A sense of purpose matters too. Many women find midlife is a time to rediscover old passions or take on new challenges. What would you love to learn, create or contribute in the years ahead?

TRY THIS WEEK

- Schedule two 15-minute strength sessions into your calendar this week, just as you would an appointment.
- Take a 10-minute walk after your largest meal of the day on at least three days.
- Set a "caffeine curfew" of 2 p.m. and notice whether your sleep changes.

CHAPTER SUMMARY

- Strength training twice a week is one of the most powerful habits for midlife, protecting muscle, bone, blood sugar and mood.
- Aim for 150 minutes of moderate cardio a week, and move more throughout the day.
- A cool bedroom, a regular schedule, a calming wind-down and limiting caffeine and alcohol all support better sleep.
- Regular moments of rest, breathing and mindfulness help manage stress.
- Alcohol can trigger hot flashes and raises breast cancer risk. Drinking less is one of the best things you can do for your health.
- Pelvic floor exercises help with bladder leaks for many women.
- Connection and purpose are vital for wellbeing at every age.

Lifestyle Checklist

- ☐ I have strength training planned on two days this week.
- ☐ I am moving for at least 150 minutes a week, in whatever way I enjoy.
- ☐ My bedroom is cool, dark and quiet.
- ☐ I go to bed and wake up at roughly the same time each day.
- ☐ I have my last caffeine before early afternoon.
- ☐ I have several alcohol-free days each week.
- ☐ I am practicing pelvic floor exercises.
- ☐ I have at least one moment of rest or relaxation each day.
- ☐ I have connected with a friend or loved one this week.

PART THREE

Nourishing Your Changing Body

Food will not fix everything, but it can help with almost everything. Here is how to make it work for you.

CHAPTER 8

Not Sure of Your Stage? Start Here

Wherever you are in the transition, there is a simple place to begin.

Welcome to the nutrition part of the book. If you have read Part One, you will already have a good sense of where you are in the menopause transition. If you have jumped straight here, that's fine too. This short chapter will help you work out your stage and point you to the parts of the book that will help you most.

Which Stage Am I In?

Answer these three questions:

1. **Have you gone 12 months or more without a period** (without another explanation, such as pregnancy, a hormonal IUD or certain medications)?
    - Yes: you are **post-menopausal**.
    - No: go to question 2.
2. **Have your periods changed** in timing, flow or frequency, or have you started to notice symptoms such as hot flashes, night sweats, disrupted sleep or new anxiety?
    - Yes: you are probably in **perimenopause**.
    - No: you may not have started the transition yet. The habits in this book will still give you a great foundation.
3. **Have you had a hysterectomy, use a hormonal IUD, or had your ovaries removed?** It may be harder to tell your stage from periods alone. Use your symptoms and age as your guide, and talk to your doctor. If both ovaries have been removed, you are in surgical menopause.

The year or so either side of your final period is often called the "menopause" stage in this book, because many symptoms peak around this time. Don't

worry about labeling yourself exactly. The stages blend into one another, and the core principles of eating well are the same throughout.

Your Stage at a Glance

	Perimenopause	Around menopause	Post-menopause
Main focus	Steady energy, mood, sleep and cycle changes	Hot flashes, sleep, joints, body composition	Long-term bone, heart, muscle and brain health
Priority nutrients	Iron (if periods are heavy), B vitamins, magnesium, protein, fiber	Protein, calcium, vitamin D, fiber, phytoestrogens	Protein, calcium, vitamin D, vitamin B12, omega-3s
Common symptoms to target	Energy dips, cravings, anxiety, PMS, bloating	Hot flashes, night sweats, sleep, joint aches	Lingering hot flashes, joint stiffness, dry skin
Foods to emphasize	Lentils, beans, leafy greens, lean meat or tofu, whole grains, nuts and seeds	Soy foods, flaxseed, dairy or fortified alternatives, oily fish, colorful vegetables	Oily fish, dairy or fortified alternatives, eggs, legumes, berries, nuts, olive oil
Iron needs	Around 18 mg a day	Around 18 mg until periods stop	Around 8 mg a day
Calcium needs	Around 1,000 mg a day	Around 1,200 mg a day from age 51	Around 1,200 mg a day

Meal Timing and Hydration at Each Stage

The basic rhythm that suits most women at every stage is simple: **three balanced meals a day, with a snack if you need one.** Regular meals help keep blood sugar steady, reduce cravings and support better energy and mood. A few stage-specific tips:

- **Perimenopause.** Don't skip breakfast if you tend to crash mid-morning. Include protein at every meal to steady blood sugar. If you have premenstrual cravings, plan a satisfying snack for the late afternoon rather than relying on willpower.
- **Around menopause.** Keep evening meals moderate in size and finish eating two to three hours before bed if night sweats are a problem.

Notice whether hot drinks, spicy food or alcohol in the evening trigger hot flashes.

- **Post-menopause.** Make sure each meal contains a good portion of protein (around 25 to 30 grams) to protect muscle. Appetite can decrease with age, so if you are eating less, make every bite nutritious.

Hydration matters at every stage, and even more so if hot flashes and night sweats make you lose fluid. The US National Academies suggest that women need around 2.7 liters (about 11 cups) of total water a day from all drinks and foods, with roughly a fifth usually coming from food. In practice, aim for six to eight glasses of fluid a day, more in hot weather or when you exercise. Water, milk, herbal teas and even tea and coffee all count. Pale yellow urine is a good sign that you are drinking enough.

Where to Go Next

If you want to...	Go to...
Learn how to build balanced, satisfying meals	Chapter 9: The Menopause Plate
Find food strategies for a specific symptom	Chapter 10: Eating for Your Symptoms
Understand weight and body changes, kindly	Chapter 11: Rethinking Weight in Midlife
Make healthy eating work on a busy schedule	Chapter 12: The Busy Woman's Kitchen
Start cooking right away	Part Four: The Recipes
Follow a ready-made plan	Part Five: The 12-Week Meal Plan

CHAPTER SUMMARY

- If you haven't had a period for 12 months, you are post-menopausal. If your periods or symptoms are changing, you are probably in perimenopause.
- The core principles of eating well stay the same at every stage. Only the emphasis shifts.
- Three balanced meals a day, with a snack if needed, suits most women.
- Aim for six to eight glasses of fluid a day, more if you have night sweats or exercise.

CHAPTER 9

The Menopause Plate: Your Everyday Framework

You don't need to count every calorie or follow complicated rules. You need a simple picture you can bring to every meal.

There is no single "menopause diet," and you won't find a list of forbidden foods in this chapter. Instead, you will find a flexible framework that works for any cuisine, budget or dietary preference, and that brings together the nutrients your body needs most during this time of life.

We call it the **Menopause Plate**.

If Food Has Felt Complicated: A Gentle Note Before We Begin

Many women arrive at midlife with decades of diet messages behind them: low-fat, low-carb, points, shakes, cleanses, "clean eating." For some, food has become tangled up with guilt, anxiety or a sense of failure. If that is you, please know that this chapter is not another set of rules to get right or wrong.

The Menopause Plate is a guide, not a test. Some meals will fit it beautifully; others won't, and that's completely fine. A slice of birthday cake, a takeout pizza with friends or a bowl of your grandmother's pasta is part of a full and happy life. What matters is the overall pattern of your eating over weeks and months, not any single meal.

If you have a history of disordered eating, or find that thinking about food causes you significant distress, please consider talking to your doctor or a registered dietitian, who can offer personalized support.

The Plate Method

Picture a standard dinner plate, about 9 to 10 inches (23 to 25 cm) across, and fill it like this:

Half vegetables, a quarter protein, a quarter fiber-rich carbs, plus healthy fats

- **Half the plate: vegetables (and some fruit).** Choose a variety of colors: leafy greens, broccoli, peppers, carrots, tomatoes, cauliflower, zucchini, mushrooms, beets and more. Fresh, frozen and canned all count. Vegetables provide fiber, vitamins, minerals and protective plant compounds, and they fill you up for very few calories.
- **A quarter of the plate: protein.** Fish, seafood, chicken, turkey, eggs, lean meat, tofu, tempeh, edamame, beans, lentils, Greek yogurt or cottage cheese.
- **A quarter of the plate: fiber-rich carbohydrates.** Whole grains (such as brown rice, quinoa, oats, barley, whole-wheat pasta or bread), starchy vegetables (potatoes with their skins, sweet potatoes, corn, squash), or beans and lentils, which count as both protein and carbohydrate.
- **Plus healthy fats.** A drizzle of extra virgin olive oil, a sprinkle of nuts or seeds, a few slices of avocado, or the natural fats in oily fish.

Add flavor freely with herbs, spices, garlic, ginger, citrus, vinegar and chili. Drink water or another unsweetened drink alongside.

Plate section	Why it matters in menopause	Examples
Vegetables and fruit (half)	Fiber, antioxidants, potassium, folate; supports gut, heart and weight	Spinach, kale, broccoli, peppers, tomatoes, berries, carrots, bok choy
Protein (a quarter)	Protects muscle and bone, steadies blood sugar, keeps you full	Salmon, chicken, eggs, tofu, lentils, Greek yogurt, edamame
Fiber-rich carbs (a quarter)	Steady energy, fiber, B vitamins	Oats, quinoa, brown rice, sweet potato, whole-grain bread, beans
Healthy fats (a little)	Heart and brain health, absorption of vitamins A, D, E and K	Olive oil, avocado, walnuts, almonds, flaxseed, chia seeds

This plate works for breakfast and lunch too, although the proportions will look a little different. A breakfast might be Greek yogurt (protein) with berries (fruit), oats (fiber-rich carbs) and walnuts (healthy fats). A packed

lunch might be a grain bowl, a hearty soup with beans, or a wrap filled with protein and salad.

Protein: Your Midlife Power Nutrient

If there is one nutrient to prioritize during and after the menopause transition, it is protein. Protein provides the building blocks for muscle, bone, skin, hair, enzymes and hormones. It is also the most filling of all the nutrients, helping you feel satisfied for longer and keeping blood sugar steady.

As estrogen declines, it becomes harder for the body to maintain muscle. Getting enough protein, especially alongside strength training, is one of the most effective ways to protect your muscle, metabolism and long-term independence.

How much, how to spread it across the day, plant and animal sources

The official minimum recommendation for adults is 0.8 grams of protein per kilogram of body weight per day. Many experts in healthy aging now suggest that adults over 50 may benefit from more, around **1.0 to 1.2 grams per kilogram per day**, and possibly higher for those who are very active. For a woman weighing 150 pounds (68 kg), that is roughly 68 to 82 grams of protein a day. (If you have kidney disease, check with your doctor before increasing your protein intake.)

Just as important is **how you spread it out.** Many of us eat little protein at breakfast, a little more at lunch and most of it at dinner. Research suggests that muscles respond best when protein is spread more evenly, with around **25 to 30 grams per meal**.

Food	Approximate protein
3.5 oz (100 g) cooked chicken breast	31 g
3.5 oz (100 g) cooked salmon	25 g
3/4 cup (170 g) plain Greek yogurt	15 to 17 g
1/2 cup (113 g) cottage cheese	12 to 14 g
2 large eggs	12 g
3.5 oz (100 g) firm tofu	12 to 17 g

Food	Approximate protein
1 cup (155 g) shelled edamame	17 to 18 g
1/2 cup (100 g) cooked lentils	9 g
1/2 cup (85 g) cooked chickpeas or black beans	7 g
1 cup (240 ml) soy milk	7 to 8 g
1 oz (28 g) almonds or pumpkin seeds	6 to 9 g
2 tbsp peanut butter	7 g

Values vary by brand and preparation; always check the label.

Plant proteins such as tofu, tempeh, edamame, beans, lentils, nuts and seeds are excellent choices that also provide fiber and other beneficial nutrients. If you eat mainly or only plants, include a variety of protein sources across the day, and consider soy foods in particular, which contain all the essential amino acids.

> **PROTEIN AT BREAKFAST**
>
> - Choose one high-protein breakfast this week: Spinach & Feta Egg Muffins, the Greek Yogurt Power Bowl, Turmeric Tofu & Veggie Scramble or Golden Flax & Berry Overnight Oats made with Greek yogurt or soy milk.
> - Notice how your energy and appetite feel through the morning.

Fiber and the Gut-Hormone Connection

Fiber is the part of plant foods that your body can't fully digest. Instead, it travels to your large intestine, where it feeds the trillions of beneficial bacteria that make up your **gut microbiome**. Fiber supports:

- regular bowel movements and less constipation
- steady blood sugar, by slowing the absorption of carbohydrates
- healthy cholesterol levels (especially soluble fiber from oats, barley, beans and lentils)
- fullness and appetite control
- a diverse, healthy gut microbiome

Most adults eat far less fiber than recommended. Aim for **25 grams or more a day**. The easiest way to get there is to include vegetables, fruit, whole grains, beans, lentils, nuts or seeds at every meal. If you are not used to much fiber, increase it gradually over a few weeks and drink plenty of water, to avoid bloating.

A helpful goal is to aim for **30 different plant foods a week**. Research from the American Gut Project found that people who ate this many different plants had more diverse gut bacteria. Every fruit, vegetable, whole grain, bean, lentil, nut, seed, herb and spice counts, so it is easier than it sounds.

Meet the estrobolome: how your gut helps process estrogen

Here is something fascinating. Some of the bacteria in your gut produce enzymes that help regulate how much estrogen is recycled back into the bloodstream or removed from the body. Scientists call this collection of bacteria the **estrobolome** (emerging). A healthy, diverse microbiome appears to support healthy estrogen balance, while an imbalanced one may affect it.

Research in this area is still at an early stage, and we can't yet say exactly how to "optimize" the estrobolome. But we do know that the gut microbiome thrives on a varied, fiber-rich diet with plenty of plants, and that fermented foods such as yogurt, kefir, kimchi, sauerkraut and miso can increase microbial diversity. Gut bacteria also play a role in converting soy isoflavones into a compound called equol, which may explain why some women benefit more from soy than others.

Healthy Fats and Omega-3s

Fat is essential. It helps your body absorb vitamins A, D, E and K, supports brain and hormone health, and makes food satisfying and delicious. The key is choosing mostly unsaturated fats:

- **Monounsaturated fats:** extra virgin olive oil, avocado, almonds, cashews, peanuts and their butters.
- **Omega-3 fats:** oily fish (salmon, sardines, mackerel, trout, herring, anchovies), walnuts, ground flaxseed, chia seeds, hemp seeds and soy foods.

Omega-3 fats support heart health and are an important part of the brain. Aim to eat **oily fish twice a week** if you eat fish. If you don't, include plant sources of omega-3 daily. The plant form (ALA) is converted only partly into the forms found in fish (EPA and DHA), so vegetarians and vegans may wish to talk to a doctor or dietitian about an algae-based omega-3 supplement.

It is wise to limit **saturated fats**, found mainly in fatty and processed meats, butter, cream, cheese, coconut oil, pastries and fried foods, since they can raise LDL cholesterol, which tends to increase after menopause. You don't need to avoid them completely: just make unsaturated fats your everyday choice.

Phytoestrogens: Soy, Flax and Legumes, What the Evidence Says

Phytoestrogens are natural plant compounds with a structure similar to estrogen. They can bind weakly to estrogen receptors in the body. The two main types are:

- **Isoflavones**, found mainly in soy foods such as tofu, tempeh, edamame, soy milk, miso and soybeans, and in smaller amounts in chickpeas and other legumes.
- **Lignans**, found in flaxseed (especially ground flaxseed), sesame seeds, whole grains and some fruits and vegetables.

Phytoestrogens are much weaker than the body's own estrogen and do not work like hormone therapy. Research on hot flashes is promising but mixed (emerging): some women notice a modest improvement after several weeks of eating soy foods regularly, while others notice no difference. Two to three servings of soy foods a day is a typical amount used in studies, for example a cup of soy milk, half a cup of edamame and a portion of tofu.

Soy and breast cancer: clearing up the confusion

Many women have been told to avoid soy because of fears about breast cancer. Here is what the research now shows (established):

- In large population studies, eating soy foods is **not linked to a higher risk of breast cancer**. In Asian populations with high lifelong soy intakes, soy appears to be associated with a lower risk.

- For women who have had breast cancer, studies suggest that eating soy foods does not increase the risk of recurrence, and may be linked to a lower risk.
- Organizations such as the American Institute for Cancer Research and the American Cancer Society consider moderate amounts of soy foods safe for women, including breast cancer survivors.

The research applies to whole soy foods, such as tofu, edamame, tempeh and soy milk. Concentrated soy isoflavone supplements are a different matter, and women with a history of hormone-sensitive cancer should discuss them with their oncology team before using them.

Key Micronutrients

Vitamins and minerals are needed in small amounts but play big roles. These are the ones most important during and after the menopause transition.

Calcium, vitamin D, magnesium, iron, B vitamins: food-sources table

Nutrient	Daily need (women)	Why it matters	Top food sources
Calcium	1,000 mg (to age 50); 1,200 mg (51+)	Bone strength, muscle and nerve function	Milk, yogurt, cheese, calcium-set tofu, fortified plant milks, canned sardines or salmon with bones, kale, bok choy, almonds
Vitamin D	600 IU (to age 70); 800 IU (71+)	Calcium absorption, bone and muscle health	Sunlight on skin, oily fish, egg yolks, fortified milks and cereals; supplements often advised
Magnesium	Around 320 mg	Muscle relaxation, sleep, blood sugar, bone	Pumpkin seeds, almonds, cashews, spinach, black beans, edamame, whole grains, dark chocolate
Iron	18 mg (while periods continue); 8 mg (after menopause)	Energy, oxygen transport, concentration	Lean red meat, poultry, fish, lentils, beans, tofu, fortified cereals, spinach (with vitamin C)
Vitamin B12	2.4 mcg	Energy, nerves, red blood cells	Fish, meat, eggs, dairy, fortified plant milks, nutritional yeast

Nutrient	Daily need (women)	Why it matters	Top food sources
Folate	400 mcg	Cell repair, mood, heart health	Leafy greens, beans, lentils, asparagus, avocado, fortified grains
Potassium	Around 2,600 mg	Healthy blood pressure	Beans, lentils, potatoes, sweet potatoes, bananas, spinach, yogurt

Daily needs are based on US National Institutes of Health recommendations for adult women and may differ slightly by country.

If you eat a varied diet based on the Menopause Plate, you will meet most of your needs from food. Vitamin D is the main exception for many people, especially in autumn and winter or if you spend little time outdoors. Women who follow a vegan diet should take a vitamin B12 supplement. Ask your doctor whether a blood test or supplement is right for you.

Anti-Inflammatory Eating Made Simple

Inflammation is a normal part of the body's defense system. But low-grade, long-term inflammation is linked with heart disease, type 2 diabetes, joint pain and other conditions that become more common after menopause.

An anti-inflammatory eating pattern is not a special diet. It is, in fact, very similar to the Mediterranean diet, one of the best-researched eating patterns in the world:

- **Eat more:** vegetables, fruit (especially berries), beans and lentils, whole grains, nuts and seeds, extra virgin olive oil, oily fish, herbs and spices (such as turmeric, ginger, garlic and cinnamon), green tea.
- **Eat less:** ultra-processed foods, processed meats, refined grains, sugary drinks and sweets, fried foods, and excess alcohol.

You don't have to be perfect. Every time you add a vegetable, swap a sugary drink for water or choose olive oil, you move your overall pattern in the right direction.

Portion Guidance with the Hand Method (No Scales Needed)

Measuring and weighing food can feel time-consuming and, for some women, stressful. Your hand offers a quick, portable guide that is naturally scaled to your body size.

Food group	Your hand	Roughly equals	Per meal
Protein	1 to 1.5 palms (thickness and size of your palm)	3 to 5 oz (85 to 140 g) meat, fish or tofu	1 to 1.5 portions
Vegetables	2 fists	About 2 cups	2 or more
Fiber-rich carbs	1 cupped hand	About 1/2 to 2/3 cup cooked grains or starchy vegetables	1 to 2 portions, depending on hunger and activity
Healthy fats	1 thumb	About 1 tbsp oil, nut butter, nuts or seeds	1 to 2 portions

These are starting points, not strict limits. If you are very active, hungry or trying to build muscle, you may need more. Pay attention to your hunger and fullness signals: aim to finish meals feeling comfortably satisfied, not stuffed.

Foods to Lean Into: Quick-Reference Chart

Food	Why it helps	Easy ways to enjoy it
Oily fish	Omega-3s, protein, vitamin D	Sheet-Pan Salmon, sardines on rye toast, smoked salmon bites
Soy foods	Protein, calcium (if calcium-set), isoflavones	Tofu scramble, edamame snacks, soy milk in oats
Beans and lentils	Protein, fiber, folate, magnesium	Dal, chili, soups, salads, hummus
Leafy greens	Calcium, folate, magnesium, vitamin K	Stir into soups, stews, eggs and smoothies
Berries	Fiber, antioxidants	Overnight oats, yogurt bowls, frozen as a snack
Greek yogurt and kefir	Protein, calcium, probiotics	Breakfast bowls, dips, smoothies, sauces

Food	Why it helps	Easy ways to enjoy it
Ground flaxseed	Lignans, omega-3s, fiber	Sprinkle on oats, yogurt and salads; add to baking
Nuts and seeds	Healthy fats, magnesium, protein	Small handful as a snack; toasted on salads
Whole grains	Fiber, B vitamins, steady energy	Oats, quinoa, brown rice, barley, rye bread
Extra virgin olive oil	Heart-healthy fats, antioxidants	Dressings, roasting, drizzling
Colorful vegetables	Fiber, vitamins, protective plant compounds	Roast a tray at the start of the week
Herbs and spices	Flavor without salt; anti-inflammatory compounds	Turmeric, ginger, garlic, cinnamon, cumin

CHAPTER SUMMARY

- The Menopause Plate is a flexible guide: half vegetables, a quarter protein, a quarter fiber-rich carbs, plus a little healthy fat.
- Protein is your midlife power nutrient. Aim for around 25 to 30 grams at each meal.
- Aim for 25 grams or more of fiber a day and 30 different plant foods a week to support your gut and your estrobolome.
- Choose mostly unsaturated fats, and eat oily fish twice a week or include plant omega-3s daily.
- Soy foods are safe for most women and may modestly help hot flashes for some.
- Calcium, vitamin D, magnesium, iron, B12 and folate are key micronutrients; vitamin D often needs a supplement.
- Use your hand as a portable portion guide, and listen to your hunger and fullness.

CHAPTER 10

Eating for Your Symptoms

Food can't make every symptom disappear, but small, targeted changes can take the edge off many of them.

This chapter is designed to be used as a reference. Each section focuses on a common symptom of the menopause transition and explains, in plain language, how food and daily habits may help, what may make things worse, and one or two simple actions to try. You will also find suggestions for recipes from Part Four that fit each symptom.

A few things to keep in mind:

- **Try one change at a time** for two to four weeks, and keep notes in your symptom tracker (Appendix A). That way you will know what actually helps you.
- **Triggers are personal.** Something that sets off hot flashes in one woman may make no difference to another.
- **Food works alongside other support,** not instead of it. If a symptom is significantly affecting your life, talk to your doctor about all your options (Chapter 6).

Hot Flashes and Night Sweats

Hot flashes are caused by changes in the brain's temperature control center as estrogen fluctuates and falls (Chapter 2). Diet can't switch them off, but it can help reduce triggers and support your body overall.

Common triggers (alcohol, caffeine, spicy food, large late meals) and cooling foods

Many women notice that certain foods and drinks make hot flashes more likely or more intense. The most common culprits are:

- **Alcohol**, especially red wine, and particularly in the evening
- **Caffeine**, in coffee, tea, cola, energy drinks and chocolate

- **Spicy foods**, such as chili, hot sauce and cayenne
- **Hot drinks and very hot foods**
- **Large meals**, especially late in the evening
- **Sugary foods and refined carbohydrates**, which can cause blood-sugar swings

What may help:

- **Eat regular, balanced meals** with protein, fiber and healthy fats to keep blood sugar steady.
- **Keep evening meals moderate,** and finish eating two to three hours before bed if night sweats disturb your sleep.
- **Stay well hydrated,** particularly after a hot flash or night sweat. Keep a bottle of cold water with you.
- **Choose cooling foods and drinks** when you feel warm: cucumber, watermelon, berries, salads, chilled soups, yogurt and iced herbal teas.
- **Include soy foods and ground flaxseed regularly.** Some women find that two to three servings of soy foods a day modestly reduce hot flashes over several weeks (emerging).
- **Reach and maintain a weight that feels healthy for you.** Some research suggests that weight loss in women with a higher body weight may reduce hot flashes, although this is not the case for everyone.

> **TRY THIS WEEK**
>
> - Keep a "trigger diary" for two weeks. Note each hot flash and what you ate or drank in the hour before. Look for patterns.
> - Swap your evening glass of wine for a Tart Cherry Evening Spritz or Cucumber-Mint Cooling Water on at least three nights.

Recipes to try: Turmeric Tofu & Veggie Scramble, Sesame Edamame Noodle Bowl, Miso-Glazed Tofu Buddha Bowl, Ginger-Garlic Tofu & Vegetable Stir-Fry, Chili-Lime Edamame (go easy on the chili), Cucumber-Mint Cooling Water.

Sleep

Poor sleep is one of the most common and exhausting parts of the transition. Night sweats, anxiety, needing the bathroom and lighter sleep all play a part. Your evening routine, including what and when you eat, can help.

Evening eating, magnesium and tryptophan-rich foods, caffeine timing

- **Set a caffeine curfew.** Caffeine can stay in your system for many hours. Try to have your last caffeinated drink before about 2 p.m., and remember that green tea, black tea, cola and dark chocolate contain caffeine too.
- **Limit alcohol.** It may help you fall asleep, but it disrupts sleep later in the night and can trigger night sweats.
- **Get the timing right.** A very large meal close to bedtime can disrupt sleep, but going to bed hungry can too. Finish your main meal two to three hours before bed. If you need something later, choose a small snack.
- **Include tryptophan-rich foods.** Tryptophan is an amino acid the body uses to make serotonin and melatonin, which help regulate sleep. It is found in turkey, chicken, fish, eggs, dairy, tofu, oats, nuts and seeds. Eating it with some carbohydrate may help it reach the brain.
- **Get enough magnesium.** Magnesium plays a role in muscle relaxation and the nervous system. Pumpkin seeds, almonds, spinach, black beans and whole grains are good sources. Evidence for magnesium supplements and sleep is limited, so food is a good place to start.
- **Consider tart cherries.** Tart cherries contain small amounts of natural melatonin. A few small studies suggest tart cherry juice may modestly improve sleep in some adults (emerging). Choose unsweetened juice and keep portions small.
- **Watch late-evening fluids.** Drink well during the day, but taper off in the hour or two before bed to reduce night-time bathroom trips.

Good bedtime snacks: a small pot of Greek yogurt with a sprinkle of oats, a slice of whole-grain toast with almond butter, a few walnuts with a small banana, a warm mug of milk or soy milk, or a Golden Turmeric Latte made without coffee.

> **TRY THIS WEEK**
>
> - Set a 2 p.m. caffeine curfew for one week and note any change in your sleep.
> - Try a small, calming bedtime snack instead of going to bed hungry or overfull.

Recipes to try: Baked Cinnamon Pear with Walnuts, Vanilla Chia Pudding Cups, Tart Cherry Evening Spritz, Golden Turmeric Latte, Greek Turkey Meatballs with Tzatziki, Three-Bean Vegetable Chili.

Mood and Anxiety

Changes in mood are very common during the transition (Chapter 5). While food can't replace professional support when it is needed, it can make a meaningful difference to how resilient you feel day to day.

Blood-sugar balance, omega-3s, the gut-brain axis

- **Keep blood sugar steady.** Long gaps between meals, or meals based mainly on refined carbohydrates, can lead to blood-sugar dips that show up as irritability, anxiety and low mood. Eat regular meals that combine protein, fiber and healthy fats.
- **Eat oily fish twice a week.** Omega-3 fats, particularly EPA, are important for brain health, and some studies suggest they may help with depressive symptoms (emerging). Plant sources include walnuts, chia seeds and ground flaxseed.
- **Look after your gut.** The gut and brain communicate constantly through nerves, hormones and the immune system, a connection known as the **gut-brain axis**. Most of the body's serotonin is actually made in the gut. A varied, fiber-rich diet with fermented foods supports a healthy gut microbiome, which may in turn support mood (emerging).
- **Follow a Mediterranean-style pattern.** Several studies have linked Mediterranean-style eating with a lower risk of depression, and one well-known trial found that improving diet quality helped symptoms in people with depression (emerging).
- **Get enough folate, B12 and vitamin D.** Low levels of these nutrients have been linked to low mood.
- **Be mindful of alcohol and caffeine.** Alcohol is a depressant, and too much caffeine can heighten anxiety and trigger palpitations.

TRY THIS WEEK

- Make sure you never go more than four to five hours without eating during the day this week.
- Add one portion of oily fish, or a tablespoon of ground flaxseed or chia seeds, to your meals on three days.

Recipes to try: Sheet-Pan Salmon with Broccoli & Sweet Potato, Smoked Salmon & Avocado Rye Toast, Golden Flax & Berry Overnight Oats, Walnut-Date Energy Bites, Kefir Berry Shake.

Energy and Fatigue

Feeling drained is one of the most common complaints in midlife. Poor sleep, stress, heavy periods and changes in blood sugar all play a part. If you feel exhausted all the time, see your doctor to rule out conditions such as anemia, thyroid problems, low vitamin D or B12, diabetes and depression.

Iron, B12, protein at breakfast, hydration

- **Start the day with protein.** A breakfast with around 25 to 30 grams of protein helps keep energy steady through the morning and reduces mid-morning cravings.
- **Check your iron.** If your periods are heavy, you may be low in iron even if you are not yet anemic. Ask your doctor to check your ferritin. Include iron-rich foods such as lean red meat, chicken, fish, lentils, beans, tofu and fortified cereals, and pair plant sources with vitamin C to help absorption.
- **Think about B12.** Absorption declines with age and with some medications. Vegans need a reliable source, such as fortified foods or a supplement.
- **Drink enough fluid.** Even mild dehydration can make you feel tired and foggy.
- **Choose slow-release carbohydrates.** Whole grains, beans, lentils and starchy vegetables provide steady energy compared with white bread, pastries and sweets.
- **Move.** It may feel counterintuitive, but regular activity is one of the most effective ways to boost energy.

> **TRY THIS WEEK**
>
> - Have a protein-rich breakfast every day this week.
> - Fill a water bottle each morning and aim to finish it by mid-afternoon.

Recipes to try: Spinach & Feta Egg Muffins, Besan Chilla with Mint Yogurt, Green Glow Protein Smoothie, Spiced Red Lentil Dal with Spinach, Lentil Bolognese with Whole-Grain Pasta.

Cravings and Emotional Eating

Many women notice stronger cravings during the transition, especially for sweet or starchy foods in the afternoon and evening. This is not a lack of willpower.

Why they happen and kind strategies

Cravings can be driven by:

- **Blood-sugar dips** after meals that are low in protein and fiber
- **Poor sleep**, which increases the hunger hormone ghrelin and decreases the fullness hormone leptin
- **Stress**, through the effects of cortisol on appetite
- **Hormonal shifts**, including premenstrual changes in perimenopause
- **Restriction**: skipping meals or banning certain foods often makes cravings stronger
- **Emotions**: boredom, loneliness, sadness, exhaustion or the need for comfort

Kind, practical strategies:

- **Eat regular, satisfying meals.** Most cravings are much easier to manage when you are not overly hungry.
- **Build in a planned snack.** If you always crave something sweet at 4 p.m., plan for it: an apple with almond butter, a few squares of dark chocolate with almonds, or a yogurt bowl.
- **Pause before eating.** Ask yourself: "Am I physically hungry, or is something else going on?" If you are not hungry, what do you really need? Rest, a break, connection, a cry, fresh air?
- **Allow all foods.** Labeling foods as "forbidden" often makes them more tempting. Enjoy treats mindfully, without guilt, and savor them.
- **Make your environment work for you.** Keep satisfying snacks easy to reach, and less nutritious foods a little less convenient.
- **Prioritize sleep.** It is one of the most powerful tools for reducing cravings.

If emotional eating feels out of control, or food and eating cause you significant distress, please talk to your doctor or a registered dietitian. You deserve support.

TRY THIS WEEK

- Plan your afternoon snack in advance on five days this week.
- When a craving strikes, wait ten minutes and drink a glass of water. If you still want the food, enjoy it slowly and without guilt.

Recipes to try: Dark Chocolate & Almond Clusters, Apple Slices with Almond Butter & Cinnamon, Berry Greek Yogurt Bark, Banana-Oat Mini Muffins, Spiced Pumpkin Seed Trail Mix.

Brain Fog

Forgetfulness, difficulty concentrating and losing your train of thought are very common during perimenopause (Chapter 5). They usually improve with time, and good nutrition supports your brain along the way.

Omega-3s, steady blood sugar, hydration and sleep

- **Omega-3 fats** are a major structural part of the brain. Eat oily fish twice a week, or include walnuts, chia seeds and ground flaxseed daily.
- **Steady blood sugar** helps your brain, which relies on a constant supply of glucose. Regular balanced meals help prevent the dips that worsen concentration.
- **Hydration** matters: even mild dehydration can affect attention and memory.
- **Berries and leafy greens** are rich in protective plant compounds. The MIND diet, a combination of the Mediterranean and DASH diets that emphasizes these foods, has been linked with slower cognitive decline in observational studies (emerging).
- **Sleep** is when your brain clears waste and consolidates memories. Protecting your sleep is one of the best things you can do for brain fog.

TRY THIS WEEK

- Add a handful of berries and a portion of leafy greens to your day, every day this week.

Recipes to try: Golden Flax & Berry Overnight Oats, Mediterranean Lentil & Roasted Vegetable Salad, Tuna & White Bean Niçoise Salad, Sheet-Pan Salmon with Broccoli & Sweet Potato, Frozen Yogurt-Dipped Blueberries.

Bloating and Digestion

Fluctuating hormones affect the gut, so bloating, gas, constipation and a more sensitive stomach are common during the transition. Stress and changes in eating patterns can add to this.

Pacing fiber, fermented foods and common trigger foods

- **Increase fiber gradually.** Fiber is essential for digestion, but adding a lot at once can cause bloating. Increase slowly over a few weeks, and drink plenty of water to help fiber do its job.
- **Include fermented foods.** Yogurt, kefir, sauerkraut, kimchi, miso and tempeh contain live bacteria that may support gut health. Start with small amounts if you are not used to them.
- **Eat slowly and chew well.** Eating quickly means swallowing more air.
- **Move after meals.** A short walk helps digestion.
- **Notice common triggers.** Carbonated drinks, chewing gum, sugar alcohols (such as sorbitol and xylitol, found in "sugar-free" products), very fatty meals, large amounts of beans or cruciferous vegetables (if you are not used to them), alcohol and caffeine can all cause bloating in some people.
- **Consider whether a particular food group bothers you.** Some people are sensitive to certain fermentable carbohydrates known as FODMAPs (found in foods such as onion, garlic, wheat, some fruits and legumes). If you suspect this, work with a dietitian rather than cutting out lots of foods on your own.
- **Drink enough fluid** to help prevent constipation.

WHEN TO SEE A DOCTOR

See your doctor if bloating is persistent (most days for three weeks or more), especially if you also have a loss of appetite, feel full quickly, have pain in your abdomen or pelvis, need to urinate more often, or notice changes in your bowel habits, unexplained weight loss or bleeding. These symptoms are usually caused by something harmless, but persistent bloating can occasionally be a sign of ovarian or bowel cancer and should always be checked.

TRY THIS WEEK

- Add one small serving of a fermented food each day this week.
- Take a 10-minute walk after your evening meal.

Recipes to try: Ginger Brown Rice Congee with Soft Egg, Greek Yogurt Power Bowl, Miso-Glazed Tofu Buddha Bowl, Kefir Berry Shake, Golden Turmeric Latte.

Joint Aches and Stiffness

Estrogen helps reduce inflammation and maintain the tissues around your joints. As levels fall, many women experience new aches and stiffness, particularly in the morning.

Anti-inflammatory foods, omega-3s, vitamin D and gentle movement

- **Follow an anti-inflammatory eating pattern** (Chapter 9), rich in vegetables, fruit, whole grains, beans, nuts, seeds, olive oil and oily fish.
- **Include omega-3 fats** from oily fish, walnuts, flaxseed and chia seeds.
- **Use spices such as turmeric and ginger.** These contain natural compounds with anti-inflammatory properties. Research on turmeric (curcumin) supplements for joint pain is promising (emerging), but supplements can interact with some medicines, including blood thinners, so check with your doctor first. Using these spices generously in cooking is a safe, delicious start.
- **Check your vitamin D.** Low vitamin D is common and can contribute to aches and muscle weakness.

- **Keep moving.** Gentle, regular movement, such as walking, swimming, cycling, yoga and strength training, helps keep joints mobile and builds the muscles that support them.
- **Maintain a weight that feels healthy for you.** Every extra pound of body weight adds several pounds of pressure on your knees when you walk, so even small changes can ease the load.

> **TRY THIS WEEK**
>
> - Cook with turmeric, ginger or both at least three times this week.
> - Start each morning with five minutes of gentle stretching from the Pose Library in Part Six.

Recipes to try: Turmeric Tofu & Veggie Scramble, Coconut Chickpea & Vegetable Curry, Spiced Red Lentil Dal with Spinach, Baked Cod with Tomatoes, Olives & Capers, Golden Turmeric Latte.

Symptom-to-Food Quick-Reference Table

Symptom	Foods and habits that may help	Possible triggers to notice
Hot flashes and night sweats	Regular balanced meals, soy foods, flaxseed, cold water, cooling foods	Alcohol, caffeine, spicy food, hot drinks, large late meals
Sleep	Tryptophan-rich foods, magnesium-rich foods, small bedtime snack, tart cherries	Caffeine after 2 p.m., alcohol, large or spicy late meals
Mood and anxiety	Steady blood sugar, oily fish, fiber and fermented foods, Mediterranean pattern	Skipped meals, alcohol, too much caffeine
Energy and fatigue	Protein at breakfast, iron-rich foods with vitamin C, B12, fluids, whole grains	Sugary snacks, skipped meals, dehydration
Cravings	Regular meals with protein and fiber, planned snacks, good sleep	Restriction, long gaps between meals, poor sleep, stress
Brain fog	Omega-3s, berries, leafy greens, steady blood sugar, fluids	Dehydration, poor sleep, blood-sugar dips

Symptom	Foods and habits that may help	Possible triggers to notice
Bloating and digestion	Gradual fiber increase, fermented foods, fluids, walking after meals	Fizzy drinks, sugar alcohols, eating quickly, sudden fiber increases
Joint aches	Anti-inflammatory foods, omega-3s, turmeric, ginger, vitamin D, movement	Highly processed foods, inactivity

CHAPTER SUMMARY

- Food can't switch off symptoms, but targeted changes can reduce triggers and support your body.
- Test one change at a time for a few weeks, and track what works for you.
- For hot flashes, notice triggers such as alcohol, caffeine, spicy food and large late meals.
- For sleep, set a caffeine curfew, limit alcohol and get your meal timing right.
- For mood, energy, cravings and brain fog, steady blood sugar is key: eat regular meals with protein, fiber and healthy fats.
- For bloating, increase fiber gradually and include fermented foods.
- For joints, lean into anti-inflammatory foods, omega-3s and gentle movement.

CHAPTER 11

Rethinking Weight in Midlife

Your body is not failing you. It is changing, and it deserves understanding, not punishment.

Weight is one of the most emotionally charged topics for women in midlife. Many women find that their weight and shape change during the menopause transition, sometimes quite quickly, even though they haven't changed how they eat or move. It can feel confusing, frustrating and unfair.

This chapter takes a compassionate and realistic look at what is happening and what genuinely helps. You won't find crash diets, calorie targets or before-and-after photos here. You will find an approach focused on health, strength and feeling good in your body, whatever its size.

Why Weight and Shape Change During Menopause

On average, women gain around 1.5 pounds (0.7 kg) a year during midlife, although some gain much more and some none at all. Perhaps even more noticeable than the number on the scale is the change in **body composition** (the balance of muscle and fat) and **where** fat is stored.

Hormones, muscle loss, sleep, stress: it's not a lack of willpower

Several factors work together:

- **Falling estrogen** changes where the body stores fat, shifting it from the hips and thighs toward the abdomen.
- **Muscle loss.** Muscle mass declines with age, and the process tends to accelerate around menopause. Less muscle means your body burns a little less energy, and it can also affect blood-sugar control.
- **Changes in insulin sensitivity** can make it easier to store fat and harder to keep energy steady.
- **Poor sleep** increases hunger hormones, reduces fullness hormones and makes us more likely to reach for quick energy.

- **Stress** and higher cortisol levels can increase appetite and favor fat storage around the middle.
- **Life changes.** Busy schedules, caring responsibilities, less time to cook and exercise, and changes in work all play a part.
- **Medications** such as some antidepressants and steroids can contribute to weight gain.

In other words, weight change in midlife is driven by biology and life circumstances. It is not a character flaw.

Why Your Shape May Shift to the Middle

Even women whose weight stays the same often notice their waistline thickening. Before menopause, estrogen encourages the body to store fat under the skin on the hips, buttocks and thighs (subcutaneous fat). After menopause, more fat tends to be stored deep within the abdomen, around the organs. This is called **visceral fat**.

Visceral fat matters because it is metabolically active: it releases substances that can increase inflammation and insulin resistance, and it is linked with a higher risk of heart disease and type 2 diabetes. This is why waist measurement can be a useful health check. As a general guide, many health organizations suggest that a waist measurement above 35 inches (88 cm) for women is linked to increased health risks, although healthy thresholds vary by ethnicity and body frame.

The encouraging news is that visceral fat responds well to the habits in this book: regular physical activity (including strength training), a fiber-rich, plant-forward eating pattern, enough protein, good sleep, reduced alcohol and stress management. These habits can reduce visceral fat even when the scale doesn't change much.

Why Crash Diets Backfire, Especially Now

When women notice weight gain, the instinct is often to eat much less. Unfortunately, very low-calorie diets tend to backfire, and in midlife the costs can be higher:

- **They cause muscle loss.** When you eat too little, your body breaks down muscle as well as fat. Losing muscle is exactly what we want to avoid during and after menopause.
- **They can harm bone health,** particularly if calcium, vitamin D and protein intakes are low.
- **They increase hunger and cravings.** Severe restriction triggers powerful biological responses that drive you to eat.
- **They slow your metabolism** slightly as your body adapts to less food.
- **They are rarely sustainable.** Most people regain weight after a crash diet, and repeated cycles of losing and regaining can leave you with less muscle and more fat than before.
- **They take a toll on your relationship with food,** your mood and your energy.

A gentler, sustainable approach is far more effective in the long run.

Fasting, Keto and Other Trends: What the Evidence Really Says

You have probably heard about intermittent fasting, keto, carb cycling and dozens of other approaches promoted for menopause. Here is a balanced look at the most popular ones:

- **Intermittent fasting** (such as eating within an 8- to 10-hour window, or "5:2" fasting) can help some people eat less overall. However, studies generally find it works about as well as other ways of reducing calories, not better (emerging). Some women find it helpful and easy; others find it increases hunger, irritability or poor sleep, or makes it harder to get enough protein. If you try it, make sure your meals are still balanced and protein-rich.
- **Ketogenic (keto) and very low-carb diets** can lead to weight loss in the short term, but they are difficult to sustain, often low in fiber, and may be high in saturated fat, which can raise LDL cholesterol. They also limit beans, whole grains and fruit, which have well-established benefits for heart and gut health.
- "**Menopause supplements" and detox teas** for weight loss generally have little or no evidence behind them.

The best eating pattern for you is one you can follow happily and consistently, that includes enough protein, fiber and nutrients, and that

supports your health beyond the scale. For most women, the Menopause Plate (Chapter 9) offers exactly that.

GLP-1 Medications and Menopause: Eating to Protect Your Muscle

GLP-1 medications, such as semaglutide and tirzepatide, have become widely used for weight management and type 2 diabetes. They work by mimicking hormones that regulate appetite and blood sugar, helping people feel full sooner and for longer. For many people, they lead to significant weight loss and health improvements.

If you are using, or considering, one of these medications, there are a few important things to know:

- **Muscle loss is a concern.** In clinical trials, a substantial share of the weight lost on these medications (often a quarter or more) was lean mass, which includes muscle. For midlife women, who are already losing muscle more quickly, protecting it is essential.
- **Protein becomes even more important.** Because appetite is reduced, it can be hard to eat enough. Prioritize protein at every meal, aiming for around 25 to 30 grams, and eat your protein first.
- **Strength training is essential.** Resistance exercise at least twice a week helps maintain muscle and bone while you lose weight.
- **Nutrient density matters.** With smaller portions, choose foods that pack in nutrients: vegetables, fruit, beans, whole grains, dairy or fortified alternatives, fish and lean protein.
- **Stay hydrated** and increase fiber gradually to help manage common side effects such as nausea and constipation.
- **Work with your healthcare team,** including a dietitian if possible, and discuss how the medication may interact with other treatments.

Sustainable Strategies That Work

What genuinely helps women feel healthier and more comfortable in their bodies through midlife? The research consistently points to a handful of habits.

Protein, fiber, strength, sleep, gentle consistency

- **Protein at every meal.** Protein keeps you full, steadies blood sugar and protects muscle. Aim for around 25 to 30 grams per meal.
- **Fiber at every meal.** Vegetables, fruit, whole grains, beans and lentils help you feel full, support gut health and reduce visceral fat.
- **Strength training twice a week.** Building muscle improves body composition, blood sugar and metabolism, even if your weight doesn't change much.
- **Daily movement.** Walking, especially after meals, supports blood-sugar control and overall health.
- **Sleep.** Protecting sleep helps regulate hunger and fullness hormones and makes it easier to make choices you feel good about.
- **Less alcohol.** Alcohol adds calories, increases appetite, disrupts sleep and contributes to abdominal fat.
- **Fewer ultra-processed foods.** Foods that are highly processed tend to be easy to overeat. A study at the US National Institutes of Health found that people ate around 500 more calories a day on an ultra-processed diet than on a minimally processed one.
- **Mindful eating.** Eating without distractions, slowly, and paying attention to your hunger and fullness helps you enjoy food more and recognize when you have had enough.
- **Gentle consistency.** Small changes that you can maintain for years beat dramatic changes that last weeks. Aim for "most of the time," not "all of the time."

Measuring Progress Beyond the Scale

The scale tells you only one thing: your total body weight. It can't distinguish between muscle and fat, and it fluctuates from day to day with fluid, hormones, digestion and salt. If you are strength training, you may build muscle and lose fat while your weight stays the same.

Instead of relying on the scale alone, try noticing:

- **How your clothes fit,** particularly around the waist
- **Your waist measurement,** checked once a month
- **Your strength:** can you carry heavier shopping, climb stairs more easily, or do more repetitions of an exercise?
- **Your energy** through the day

- **Your sleep quality**
- **Your mood and confidence**
- **Your symptoms,** such as hot flashes, joint aches or digestion
- **Your health markers:** blood pressure, cholesterol and blood sugar
- **How you feel about food:** calmer, more relaxed and more in tune with your body

If you choose to weigh yourself, consider doing it no more than once a week, at the same time of day, and focus on the overall trend over several weeks rather than any single number. If weighing yourself causes distress, it is perfectly fine not to.

> **BODY RESPECT**
>
> Health is about much more than body size. People of all shapes and sizes can improve their health through the habits in this book. Wherever you are starting from, your body deserves care, nourishment and respect, today, not just when it looks a certain way.

> **TRY THIS WEEK**
>
> - Choose one "beyond the scale" measure to track this month, such as your waist measurement, your energy levels or how many sit-to-stands you can do in 30 seconds.
> - Eat one meal a day this week without screens, slowly, noticing the flavors and how full you feel.

CHAPTER SUMMARY

- Weight and shape changes in midlife are driven by hormones, muscle loss, sleep, stress and life circumstances, not a lack of willpower.
- Fat tends to shift to the abdomen after menopause. Visceral fat responds well to activity, fiber, protein, sleep and less alcohol.
- Crash diets cause muscle and bone loss and rarely last. Sustainable, gentle change works better.
- Fasting and keto work for some people but are not superior to other approaches. Choose a pattern you can enjoy for life.
- On GLP-1 medications, prioritize protein, strength training and nutrient-dense foods to protect muscle.
- Measure progress in many ways beyond the scale: strength, energy, sleep, waist size, mood and health markers.

CHAPTER 12

The Busy Woman's Kitchen

Healthy eating doesn't have to take hours. It just needs a little planning and a few smart habits.

You now know what your body needs. This chapter is about making it happen on real weekdays, when you are tired, short on time and juggling a dozen other things. The goal is not to spend more time in the kitchen. It is to spend the time you have more wisely.

Meal Planning in 20 Minutes a Week

A simple weekly plan is one of the most powerful tools for eating well. It saves time, money and stress, and it means you are much less likely to end up eating whatever is quickest when you are exhausted. Here is a 20-minute routine:

1. **Check your week (2 minutes).** Look at your calendar. Which evenings are busy? When will you have time to cook? Are there any social events or nights out?
2. **Choose your dinners (5 minutes).** Pick three or four main recipes for the week, including at least one batch-cook or slow-cooker meal that makes leftovers. Plan one "pantry night" with something quick, and leave room for one flexible night.
3. **Plan lunches from leftovers (3 minutes).** Most dinners in this book can become the next day's lunch, either as they are or with a simple twist.
4. **Pick two breakfasts (3 minutes).** Choose one make-ahead option (such as overnight oats or egg muffins) and one quick option for variety.
5. **Choose two or three snacks (2 minutes).**
6. **Write your grocery list (5 minutes).** Check what you already have in the fridge, freezer and pantry, then list what you need by store section to save time.

Part Five includes a ready-made 12-week meal plan with grocery lists, so you can skip the planning altogether if you prefer. Appendix B has a blank weekly planner for when you want to create your own.

Cook once, eat three times

This is the secret to eating well without spending every evening cooking. One batch of food becomes three different meals:

Cook once	Eat it again	And again
Sheet-Pan Salmon with Broccoli & Sweet Potato (dinner)	Flaked into a grain bowl with leafy greens (lunch)	Mashed with Greek yogurt and herbs on rye crackers (snack or light lunch)
Three-Bean Vegetable Chili (dinner)	Spooned over a baked sweet potato (lunch)	Wrapped in a whole-grain tortilla with avocado (quick dinner)
Lemon Herb Chicken Grain Bowl (lunch)	Extra chicken in a wrap with hummus and salad (lunch)	Stirred into soup with white beans and kale (dinner)
Spiced Red Lentil Dal with Spinach (dinner)	With brown rice and yogurt (lunch)	Thinned with stock as a warming soup (lunch)

The Menopause-Friendly Grocery Guide

A well-stocked kitchen makes healthy choices the easy choices. Use this list as a template and adapt it to your tastes, budget and dietary needs.

Printable list by store section: produce, protein, pantry, dairy and alternatives, freezer

Produce

- Leafy greens: spinach, kale, arugula, mixed salad leaves, bok choy
- Cruciferous vegetables: broccoli, cauliflower, cabbage, Brussels sprouts
- Colorful vegetables: bell peppers, carrots, tomatoes, beets, zucchini, red onions
- Aromatics: garlic, ginger, onions, scallions, fresh herbs, lemons, limes
- Starchy vegetables: sweet potatoes, potatoes, squash, corn
- Fruit: berries, apples, pears, oranges, bananas, kiwi
- Avocados

Protein

- Fish: salmon, cod, trout; canned sardines, salmon and tuna
- Poultry: chicken breasts or thighs, lean ground turkey
- Eggs
- Tofu (calcium-set if possible), tempeh, edamame
- Lean red meat (occasionally, if you eat it)

Dairy and alternatives

- Plain Greek yogurt, kefir, cottage cheese
- Milk, or fortified unsweetened soy milk (good for calcium and protein)
- Cheese, such as feta, Parmesan and ricotta, in moderate amounts

Pantry

- Whole grains: rolled oats, quinoa, brown rice, whole-grain pasta, barley, buckwheat
- Whole-grain or seeded rye bread, whole-grain tortillas, corn tortillas
- Canned beans, chickpeas and lentils; dried red lentils
- Canned tomatoes, tomato paste, low-sodium stock
- Nuts and seeds: almonds, walnuts, pumpkin seeds, chia seeds, ground flaxseed, sesame seeds
- Nut and seed butters (no added sugar)
- Extra virgin olive oil, sesame oil
- Vinegars, low-sodium soy sauce or tamari, miso paste, Dijon mustard
- Spices: turmeric, cumin, cinnamon, paprika, chili flakes, garam masala, dried oregano
- Dark chocolate (70% or higher)

Freezer

- Frozen vegetables: spinach, peas, broccoli, mixed stir-fry vegetables, cauliflower rice
- Frozen berries and mango
- Frozen edamame
- Frozen fish fillets and shrimp
- Batch-cooked soups, stews, chili and dal
- Whole-grain bread (slices toast perfectly from frozen)

Pantry, Fridge and Freezer Staples

With a few basics on hand, you can make a healthy meal in minutes, even when the fridge looks empty. Some "pantry night" ideas:

- **Bean and greens skillet:** canned white beans, frozen spinach, garlic, canned tomatoes and a drizzle of olive oil, topped with an egg or served with toast.
- **Speedy fried rice:** frozen brown rice, frozen vegetables, eggs or tofu, tamari and sesame oil.
- **Pantry pasta:** whole-grain pasta, canned tuna or chickpeas, canned tomatoes, olives, capers and frozen peas.
- **Loaded sweet potato:** microwave a sweet potato and top with canned black beans, salsa, Greek yogurt and avocado.
- **Five-minute soup upgrade:** a carton of good-quality vegetable soup with added canned lentils and a handful of frozen spinach.

Easy Additions and Swaps

Small changes add up. Instead of focusing on what to cut out, think about what you can add, and which simple swaps feel easy.

Simple swap table, e.g., white bread to seeded rye

Instead of…	Try…	Why
White bread	Seeded rye or whole-grain bread	More fiber, steadier energy
Sugary breakfast cereal	Overnight oats with berries and seeds	Protein, fiber and longer-lasting fullness
Flavored, sweetened yogurt	Plain Greek yogurt with fruit and cinnamon	More protein, less added sugar
Chips or crisps	Crispy Roasted Chickpeas or a small handful of nuts	Protein, fiber, healthy fats
Sweetened soda	Sparkling water with citrus, berries or cucumber	No added sugar; helps hydration
Creamy salad dressing	Olive oil, lemon and herbs	Heart-healthy fats
White rice	Brown rice, quinoa or half rice and half cauliflower rice	More fiber and nutrients

Instead of...	Try...	Why
Processed deli meats	Leftover roast chicken, canned salmon, hummus or egg	Less salt and processed meat
Evening glass of wine	Tart Cherry Evening Spritz or an alcohol-free alternative	Better sleep, fewer hot flashes
Afternoon cookie	Two squares of dark chocolate with almonds	Satisfies sweet cravings with added nutrients
Cream in coffee	Milk or fortified soy milk	Less saturated fat, more calcium

Easy additions: a handful of spinach in eggs, sauces and smoothies; a tablespoon of ground flaxseed on oats or yogurt; a can of beans in soups and stews; seeds sprinkled on salads; an extra vegetable with every dinner; a piece of fruit with lunch.

Batch Cooking and Prep-Ahead Routines

A little preparation at the start of the week makes weekday eating much easier. You don't need a whole afternoon: 60 to 90 minutes on a Sunday (or any day that works for you) is enough. Each week of the meal plan in Part Five includes a prep plan, but here is the basic routine:

A 60-minute prep session

1. **Turn on the oven** and roast two trays of vegetables (for example, peppers, zucchini, red onion, broccoli and sweet potato) with olive oil.
2. **Cook a pot of grains,** such as quinoa or brown rice, for the next three or four days.
3. **Prepare a protein,** such as baking chicken, a tray of Spinach & Feta Egg Muffins or a block of marinated tofu.
4. **Make one batch meal,** such as a soup, chili or dal, for lunches or a busy evening.
5. **Wash and chop** salad vegetables and store them in containers lined with paper towels.
6. **Make overnight oats** or chia pudding for two or three breakfasts.
7. **Mix a jar of salad dressing.**

8. **Portion snacks,** such as nuts, seeds or energy bites, into small containers.

Food storage and safety

- Cool cooked food quickly and refrigerate within two hours.
- Most cooked dishes keep for three to four days in the fridge.
- Label and date food in the freezer. Most soups, stews and chilis keep for around three months.
- Reheat food until it is steaming hot all the way through, and reheat rice only once.
- Defrost frozen food in the fridge overnight, not on the counter.

Hydration Made Easy

Many women find it hard to drink enough, especially when they are busy. A few simple habits help:

- Drink a glass of water when you wake up, before your morning coffee or tea.
- Keep a reusable water bottle with you and refill it throughout the day.
- Link drinking to daily habits: a glass with every meal, after every bathroom trip, or every time you check your email.
- Make water more interesting with slices of citrus, cucumber, mint, berries or ginger.
- Herbal teas, milk, soy milk, sparkling water, and even tea and coffee all count toward your fluid intake.
- Eat water-rich foods, such as soups, cucumber, watermelon, oranges, berries and tomatoes.
- Drink extra after exercise, in hot weather and after hot flashes or night sweats.

Eating Out, Travel and Social Occasions

Healthy eating should fit around your life, not the other way round. Social meals, celebrations and travel are important parts of a happy, connected life. A few tips help you enjoy them while still feeling good:

- **Look for the plate.** Wherever you are, look for a protein, plenty of vegetables and a fiber-rich carb. Most menus have something that fits.

- **Don't arrive starving.** A small snack beforehand helps you make choices you feel good about.
- **Ask for what you need.** Dressing on the side, extra vegetables, or grilled instead of fried are reasonable requests.
- **Choose your treats.** Enjoy the dish or dessert you really want, slowly, and skip the ones you don't care about.
- **Pace alcohol.** Alternate each drink with a glass of water, or try alcohol-free options.
- **When traveling,** pack snacks such as nuts, fruit, roasted chickpeas and protein bars, and look for grocery stores for simple meals.
- **Let go of guilt.** One meal or one weekend won't undo your progress. Enjoy it, and return to your usual routine at your next meal.

One Meal, Whole Family: Cooking for Everyone at Once

You do not need to cook separate meals for yourself and your family. The recipes in this book are designed to be family-friendly. A few tips help everyone enjoy the same meal:

- **Build-your-own meals** work brilliantly: tacos, fajita bowls, grain bowls and stir-fries let everyone choose their own toppings and portions.
- **Adjust portions, not recipes.** Growing teenagers or very active family members can have larger portions of grains and starchy vegetables, while you prioritize protein and vegetables.
- **Add spice at the table.** Keep chili, hot sauce and strong spices on the side for those who like them (and for those who find spicy food triggers hot flashes).
- **Serve vegetables first,** as a starter or side, when everyone is hungry.
- **Involve the family.** Children and partners who help choose and cook meals are much more likely to eat them.

Eating Well on a Budget

Nutritious food doesn't have to be expensive. Some of the healthiest foods are also among the cheapest:

- **Beans, lentils and chickpeas,** canned or dried, are inexpensive sources of protein and fiber. Dried versions are even cheaper.
- **Frozen fruit and vegetables** are just as nutritious as fresh, last for months and reduce waste.

- **Canned fish,** such as sardines, salmon and tuna, provides protein, omega-3s and, with bones, calcium.
- **Eggs and tofu** are affordable, versatile proteins.
- **Oats, brown rice and whole-grain pasta** are inexpensive staples.
- **Buy in season,** and choose store brands.
- **Plan and use leftovers** to reduce food waste, which is one of the biggest hidden costs in most households.
- **Cook in batches** and freeze portions.
- **Buy whole and prepare yourself:** a whole chicken, a bag of carrots or a block of cheese costs less than pre-cut versions.

Adapting Recipes: Gluten-Free, Dairy-Free, Vegetarian and Vegan

Every recipe in this book includes labels and swaps, so you can adapt it to your needs. These general swaps work across most recipes:

If you need...	Swap...	For...
Gluten-free	Wheat pasta, bread, couscous, tortillas	Gluten-free pasta, bread, quinoa, rice, buckwheat, corn tortillas
Gluten-free	Soy sauce	Tamari or coconut aminos
Gluten-free	Rolled oats	Certified gluten-free oats
Dairy-free	Milk	Fortified unsweetened soy, oat or almond milk
Dairy-free	Yogurt	Unsweetened soy or coconut yogurt (check for added calcium)
Dairy-free	Feta or Parmesan	Dairy-free alternatives, or nutritional yeast for a savory flavor
Vegetarian	Chicken, turkey or fish	Tofu, tempeh, beans, lentils or eggs
Vegan	Eggs (in baking)	1 tbsp ground flaxseed mixed with 3 tbsp water per egg
Vegan	Honey	Maple syrup or date syrup
Vegan	Fish sauce	Soy sauce or miso with a squeeze of lime

If you follow a vegan diet, pay particular attention to vitamin B12 (a supplement is essential), calcium, vitamin D, iron, iodine and omega-3s, and

include a variety of protein sources every day. A registered dietitian can help you plan a balanced vegan diet.

TRY THIS WEEK

- Spend 20 minutes planning next week's meals using the steps in this chapter.
- Do one 60-minute prep session, even if you only roast vegetables, cook a pot of grains and make one batch meal.

CHAPTER SUMMARY

- A 20-minute weekly plan saves time, money and stress.
- "Cook once, eat three times" turns one batch of food into several meals.
- A well-stocked pantry, fridge and freezer make healthy meals possible even on your busiest days.
- Focus on easy additions and simple swaps rather than strict rules.
- A 60-minute prep session sets you up for the week.
- Healthy eating can fit around family meals, social occasions, travel and a tight budget.
- Every recipe can be adapted to gluten-free, dairy-free, vegetarian and vegan needs.

Kitchen Checklist

- ☐ I have planned my meals for the week.
- ☐ I have a grocery list organized by store section.
- ☐ My pantry has beans, lentils, whole grains, canned fish and tomatoes.
- ☐ My freezer has vegetables, fruit, edamame and a batch-cooked meal.
- ☐ I have done a prep session for the week ahead.
- ☐ I have a water bottle ready for tomorrow.
- ☐ I have healthy snacks prepared or within easy reach.
- ☐ I know which recipes I can adapt for my family.